



Nurse Navigation

Job Training & Reference Materials

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Policies & Guidelines



About GMR



Contracts Quick Links



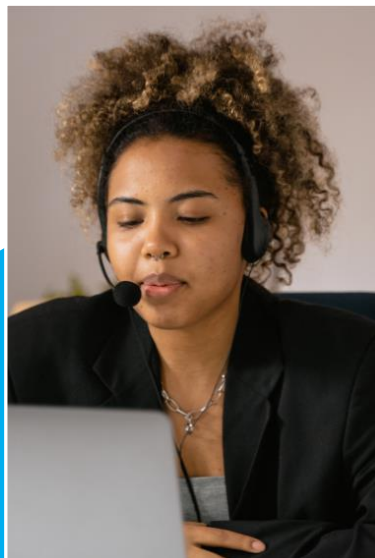
Workflows



System Applications



Training Resources





Reference Topics

- [Attendance Policy](#)
- [Breaks & Shift Relief](#)
- [Paid Time Off](#)
- [Shift Swaps & Giveaways](#)
- [Job Preparedness & Shift Restrictions](#)
- [Distractions](#)
- [Appearance](#)
- [Company-Issued ID Badge](#)
- [Lockers & Personal Belongings](#)
- [Condition & Upkeep of Facilities](#)
- [Tobacco & Smoke-Free Workplace](#)
 - [Phoenix: Tobacco & Electronic Cigarettes](#)
- [Inclement Weather](#)
- [Lactation Accommodation](#)
- [Respect in the Workplace](#)
- [Contracts, Territories, & Licensing](#)



New Hire Attestation Form

As part of your onboarding journey at Global Medical Resource (GMR), all hired employees MUST attest to their review and understanding of the policies in this section. Complete your attestation by submitting the following form:

[!\[\]\(3211b5d1d968fc1665909b34f9f16010_img.jpg\) 100-ICC People Policies Attestation NN](#)

Compliance Consideration

In addition to the policies herein, you are required to complete mandatory compliance training on a yearly basis, for more information, please reach out to your trainer or supervisor.

View your pending learning assignments and transcripts on [!\[\]\(6a9b39b98eb945faa14c645ec99e4eaa_img.jpg\) CornerStone](#).

Attendance Policy

Policy Number: 100-01	Subject: Attendance
Revision Date: January 8 th , 2026	Effective Date: January 8 th , 2026

I. PURPOSE

To establish expectations that employees report to work for all scheduled hours, including regular shifts, assigned overtime, shift swaps, giveaways, as well as mandatory meetings and classes.

II. POLICY

Expectations

- Employees are expected to arrive early enough to be performing assigned duties at the start of their shift and remain until its scheduled end time without extending allotted break periods.
- Employees may clock in no more than 10 minutes before their shift begins.
- Employees are required to be physically present in the ICC when clocking in unless they are attending a course, meeting or other business-related obligation located offsite.

III. PROCEDURE

If an employee is unable to meet the above expectations, the subsequent guidelines should be followed.

Notification Procedure

1. Employees who will be absent or anticipate being late are expected to call the Communications Center and speak with the on-duty leader on a recorded line.
 - Employees are expected to call as soon as they know they will not be reporting for duty at the start of their shift.
2. In the event an employee needs an extended break, to leave early or requires any other shift modification, they must speak with the on-duty leader in person.

- If an on-duty leader is not present in the ICC, the employee must attempt to contact the on-duty leader and receive acknowledgement of their schedule modification. Should the employee be unable to reach their on-duty leader, the matter should be escalated to another departmental ICC on-duty leader.

Attendance Record Documentation

- Attendance records are kept using a rolling twelve (12) month period, calculating attendance occurrences over the previous 365 days. Occurrences are archived and deducted from the employee's record 365 days after they occur.
- The highest level of corrective action taken during the merit period (January 1st – December 31st) will be included in an employee's annual review, regardless of the total number of occurrences on record at the conclusion of the calendar year.

Appeal Process

1. Employees who wish to contest an occurrence must do so via email to the Communications Manager within thirty (30) calendar days of the date of the occurrence in question.
2. Employees will receive a response to their appeal via email within ten (10) business days. Appeals that are not submitted as specified above, or those denied by the Manager, will become a permanent part of the employee's attendance record.

PTO Application

- PTO will be applied to all absences from an employee's regularly scheduled shift, including those documented as FMLA.
- PTO will be applied to any partial shift absence (late arrival or early departure) in one-hour increments, except in the case of Voluntary Time Off (VTO).
- PTO will not be applied to absences on overtime shifts (scheduled in addition to an employee's regular schedule).

Types of Absences

- Approved Absences - Absences arranged and authorized in advance. They may include:
 - Company observed holidays (for specified employees).
 - Pre-approved PTO, shift swaps, shift giveaways and scheduled shift modifications documented in the Company-approved scheduling system.
 - Family Medical Leave (if stated at the time of notification with an approved FMLA case).
 - Additionally, employees are required to notify GMR's third-party leave management company, in accordance with their established guidelines, of any shift, full or partial, that is missed for which FMLA has been claimed.
 - Time that is not submitted to or has been denied by GMR's third-party leave management company will result in the occurrence being issued according to this policy.
 - Approved Disability Leave.
 - Absences related to an on-the-job injury, wherein the injury has been reported to leadership, and a claim has been filed with and approved by GMR's third-party risk management company.
 - Bereavement per GMR policy with proper documentation.
 - Military Leave with proper documentation.
 - Jury Duty with proper documentation.
 - Work-related court appearances with proper documentation.
 - Any additional absence permitted under applicable law.
- Unapproved Absences - Absences not classified as "approved absences" in this policy.
- Multiple Day Absences - A subsequent unapproved absence on a consecutive calendar day will be recorded as one-half of an occurrence.
 - Unapproved absences spanning three (3) or more consecutive calendar days will result in one (1) occurrence being recorded for each shift missed after the initial two (2) absences.

Absence Exceptions

- Return to work with a provider note: Should an employee return to work with an official signed document bearing the signature of a licensed medical provider, absences of greater than one (1) shift may be considered one (1) occurrence. Documentation must be provided to leadership upon the employee's return to work.
- Return to work following hospitalization: Absences during a hospital admission will be recorded as zero occurrences if valid paperwork inclusive of date of admission, discharge and a return-to-work date are provided to leadership upon the employee's return to work.

Documentation

- Be advised that any documentation provided to leadership for the purpose of this policy is subject to verification, and any instance of falsification of documents may result in corrective action, up to and including termination.

Definitions

- Tardy: Reporting for duty one to ten (1-10) minutes after the scheduled start time of an employee's shift. Employees will be issued one-quarter of an occurrence.
- Late: Reporting for duty more than 10 minutes after the scheduled start time but not exceeding 25% of the employee's shift duration. Employees will be issued one-half of an occurrence.
- Early Departure: Leaving assigned duty prior to the scheduled end time of an employee's shift.
 - Employees who work less than 75% of their scheduled shift and leave early will be issued one occurrence.
 - Employees who worked more than 75% of their scheduled shift and leave early will be issued one-half of an occurrence.
 - Notification of early departure is to be made directly to the on-duty leader, and the employee must confirm receipt of the notification. If proper notification procedures are not followed, this may be considered job abandonment and may result in corrective action, up to and including termination.
- Voluntary Time Off (VTO): Unpaid time off during periods of low demand, as determined by an on-duty leader.

- **No Call / No Show:** Not reporting to work within two hours of a scheduled shift without proper notification to an on-duty leader, provided extraordinary circumstances do not apply.

Multiple Occurrences (same shift)

Should an employee incur a tardy or late arrival in combination with an early departure during the same scheduled shift, each instance will be documented individually; however, no more than one occurrence will be issued.

Mandatory Meetings and Classes

Required meetings in addition to mandatory training or classes are considered part of an employee's work schedule. Failure to attend these events will be recorded as an occurrence.

Occurrences of Significant Impact

Certain types of absences may have significant adverse effects on staffing plans. Such instances are particularly detrimental to operational efficiency and will result in an expedited corrective action process. The initial instance of any of the following occurrences will result in a final warning. A subsequent instance (any combination) of the occurrences listed below within a rolling 365-day period will be grounds for termination.

- Calling out on a company-observed holiday.
- Calling out for a scheduled shift for which PTO had been requested and denied.
- Calling out unable to report to work due to inclement weather after accommodations have been offered by the Company.
- No Call / No Show

Progressive Discipline

Excessive occurrence accumulation will lead to progressive discipline. "Excessive" is defined by the following criteria:

- 3 – Attendance Notification
- 4 – Verbal Warning
- 5 – Written Warning
- 7 – Final Warning
- 9 – Termination

Redemptions

Employees may earn the opportunity to have occurrences removed from their attendance record as follows:

- An employee who accepts a last-minute shift coverage request from a departmental leader may have up to one full occurrence removed from their attendance record to a maximum of one per calendar quarter.
 - Coverage of one full shift removes one occurrence.
 - Coverage of a half shift removes one-half of an occurrence.
- Shift redemptions will be forfeited if:
 - The employee has previously been offered and declined an on-call or overtime opportunity for the date in question. (Failure to respond to an overtime offer will be considered declination.)
 - The employee does not work all of their scheduled shifts within that pay period.

Employees may not have a negative attendance occurrence balance.

Breaks & Shift Relief

Policy Number: 100-02	Subject: Breaks and Shift Relief
Revision Date: January 8 th , 2026	Effective Date: January 8 th , 2026

I. PURPOSE

To establish standards for breaks as well as proper shift relief for all GMR ICC employees.

II. POLICY

Breaks

This basic outline establishes typical break periods for employees based on their total shift time. Due to variable demands in our workplace, these breaks are provided on paid company time to allow for necessary flexibility in managing fluctuating workloads.

Employees may be allotted breaks as outlined in the following chart. The specific duration and scheduling of breaks will be determined by operational leadership.

Total Shift Hours	Total Break Time
6 Hours	20 Minutes
8 Hours	40 Minutes
10+ Hours	60 Minutes

While we make every effort to provide breaks as outlined in this policy, they are not guaranteed and are contingent upon staffing, workload and operational needs of the department. Additional break expectations are as follows:

- Employees are expected to remain logged into their workstations and all software, as well as to utilize the designated telephone status codes appropriately during break periods.
- Employees must remain on-site and be reachable by telephone during break periods, able to return to the ICC immediately if needed.
- In the rare case that there is a need to leave the premises during a break, direct acknowledgment and approval from the on-duty leader must be obtained. Employees should clock out before leaving and clock in upon return.

- Employees are required to obtain direct acknowledgment and approval from the on-duty leader before leaving the room, except during regularly scheduled breaks.
- Trainees and trainers are required to take their breaks together, unless otherwise instructed to stagger them by the on-duty leader.

Shift Relief

To ensure continuous operational oversight, employees should remain logged in and available at their workstations until properly relieved by the on-coming employee taking over their assignment.

- Employees must receive the approval of the on-duty leader to end their shift if their scheduled relief is not present at the conclusion of their shift.
- Air employees without proper relief may be held over for up to six (6) hours.

Paid Time Off (PTO)

Policy Number: 100-03	Subject: Paid Time Off (PTO)
Revision Date: January 8 th , 2026	Effective Date: January 8 th , 2026

I. PURPOSE

This policy outlines procedures for requesting and approving Paid Time Off (PTO). It does not govern absences protected by federal, state, or local law, including but not limited to FMLA, military leave, jury duty, disability-related leave, or pregnancy-related leave.

II. POLICY

PTO Requests

- Should be submitted via the company-approved scheduling system and departmental process for approval.
- Will be reviewed on a first requested-first considered basis.
- Will be limited by departmental guidelines for staffing.
- May be submitted for consideration up to 90 days prior to the intended date(s).
- Must be submitted at least 30 days prior to the intended date(s).
 - Requests made less than 30 days in advance will be denied unless the requestor arranges shift coverage as outlined in the Shift Swaps and Giveaways Policy.
- Will be reviewed by a member of leadership and a response will be provided within ten (10) business days of submission when submitted per policy.
 - Responses will be designated as approved, denied or pending.
- Will be denied if the requestor will not have sufficient PTO hours to cover the intended date(s).
- For which the requestor's PTO balance is insufficient at the time of submission will remain in a pending status until the proper number of hours have accrued to cover the intended date(s).
- That have been previously approved are subject to the approval being withdrawn if the requestor's PTO balance becomes insufficient for the intended date(s). The employee will then remain responsible for their originally scheduled shift(s).
 - Employees may not take unpaid time off.

- For Company-observed holidays will be reviewed with consideration for past holiday usage.

Leadership will review all employee requests for PTO and make every effort to accommodate eligible requests. However, requests may be denied due to operational requirements, staffing levels, or other business considerations.

It is the responsibility of the employee to confirm approval for all requested PTO date(s). Should an employee be absent without approval, corrective action per attendance standards may apply.

Shift Swaps & Giveaways

Policy Number: 100-04	Subject: Shift Swaps & Giveaways
Revision Date: January 8 th , 2026	Effective Date: January 8 th , 2026

I. PURPOSE

This policy outlines procedures for employees to swap or give away shifts. It does not govern absences protected by federal, state, or local law, including but not limited to FMLA, military leave, jury duty, disability-related leave, or pregnancy-related leave.

II. POLICY

Shift Swaps

Employees are eligible to swap shifts if:

- They have been fully cleared from training.
- Both parties have equivalent title and capabilities.
- The shifts or partial shifts involved are equal in duration.
- Both shifts included in the swap must occur within the same pay period.
- Both employees acknowledge the shift swap via the department-approved process.
- The request is approved by leadership before the first shift involved begins.

Shift Giveaways

If an employee's PTO request is denied, the employee may arrange shift coverage and process such as a shift giveaway if:

- The accepting employee has an equivalent title and capabilities.
- When giving away an overtime assignment, the receiving employee must meet the shift's minimum requirements, not the title and capabilities of the employee giving the shift away.
- They have sufficient PTO available to cover the shift they intend to give away if it is a regularly scheduled shift.
 - If the shift giveaway applies to an overtime assignment that the requesting employee initially accepted, the use of PTO is not required.
- The request is approved by leadership before the shift begins.

Once approved, the covering employee assumes responsibility for the shift under attendance standards.

Job Preparedness and Shift Restrictions

Policy Number: 100-05	Subject: Job Preparedness and Shift Restrictions
Revision Date: November 19 th , 2025	Effective Date: January 1 st , 2026

I. PURPOSE

To establish standards for employees to work efficiently, provide excellent service and ensure high-quality performance.

II. POLICY

Employees are required to arrive well-rested and capable of fulfilling the needs of the public and our patients. Maintaining alertness fosters a safe workplace and upholds high standards of service and patient care. Fatigue can negatively impact individuals and may endanger both patients and colleagues. The use of drugs or alcohol, as well as sleeping during work hours, is strictly prohibited. Employees who are found to be in potential violation of these standards may be sent home without pay and could be subject to corrective action up to and including termination.

In some situations, an employee may be required to remain at work beyond their scheduled shift. The maximum permitted shift length is eighteen (18) hours.

Distractions

Policy Number: 100-06	Subject: Distractions
Revision Date: N/A	Effective Date: January 1 st , 2026

I. PURPOSE

To establish standards to mitigate distractions in the workplace.

II. POLICY

Personal Electronic Devices

Personal electronic devices are allowed on the communications center floor provided:

- They do not inhibit the ability to perform work duties.
- They remain in silent mode.

The following items/activities are strictly prohibited:

- Smart glasses.
- Earbuds or any listening device that isn't a company-issued headset.
- Media Capture, Recording, & Streaming devices.
- Making or receiving phone calls or voice messages with a personal electronic device.
- The use of cell phones in any capacity by trainees or trainers while they are training.

If an employee chooses to take their break at their workstation and must utilize their cell phone to make or receive a call they are required to do so outside of the ICC.

Use of Company Equipment

Televisions in the ICC are the property of GMR. Employees should not request special viewing of programming that does not involve local, state or national news events and/or weather updates.

Use of Company equipment, including but not limited to computers, phones, email and messaging, for personal purposes is prohibited. There is no reasonable expectation of privacy for the use of Company equipment. The Company may access and monitor all employee electronic usage, such as computer applications, internet, voicemail, email and messages.

The Company will not exercise its rights as described for the purpose of investigating whether employees have engaged in activity protected by the National Labor Relations Act.

Streaming

Employees are prohibited from streaming or viewing non-work-related videos or music on Company or personal electronic devices within the ICC.

Socialization

Casual conversation is encouraged but must not occur across the room or distract from assigned duties. Employees should be mindful of noise levels and always avoid unprofessional conversation or language. Employees should refrain from visiting others' workstations or departments, even during breaks.

Gossip and rumors has a way of spreading negativity and creating unnecessary tension. To maintain a harmonious and productive environment, it's essential to focus on clear, direct communication and to address any concerns or questions openly. By fostering a culture of respect and transparency, we can ensure that everyone feels valued and understood, paving the way for a more collaborative and supportive workplace.

Appearance

Policy Number: 100-07	Subject: Appearance
Revision Date: January 8 th , 2026	Effective Date: January 8 th , 2026

I. PURPOSE

To establish guidelines for dress code and overall appearance expectations for employees.

II. POLICY

A casual professional dress code is permitted, although leadership may require more professional attire when certain business needs arise. Employees are expected to maintain a well-groomed and professional appearance while at work or attending business-related meetings, courses, conferences and other events.

Company-Issued Uniform Components

- Photo ID Badge
- This access card will be issued to all employees and must be displayed at all times.
- See policy 100-08 for additional details on this item.
- Shirts
- GMR polos will be issued to each employee at no cost.
- Outerwear
- One (1) GMR jacket will be issued to each employee at no cost.

Employee Responsibility Uniform Components

- Headwear
 - Head scarves or other hair accessories should be solid white, tan/khaki, gray, dark blue or black.
 - Hats must be baseball style/brimmed and bear a GMR-affiliated logo or be solid white, tan/khaki, gray, dark blue or black and free of other logos. Hats must be worn with the brim facing forward.
 - Sunglasses may not be worn.

- Undershirts
 - May be short or long sleeved
 - If worn, any visible part of the shirt should be solid white, tan / khaki, gray, dark blue or black.
- Outerwear
 - Employees may also choose to wear GMR branded jackets purchased from the GMR Brand Store provided that they are gray, dark blue, or black.
- Pants / Skirts
 - Employees can wear pants/skirts of their own choosing within the guidelines below.
 - Pants / Skirts should be solid white, tan/khaki, gray, dark blue or black.
 - While sitting, the skirt hem should remain at knee length.
 - Denim is allowed, but jeans must be in good condition, regardless of current trends.
 - No holes, faded, frayed or distressed denim.
 - No ill-fitting or “sagging” jeans.
 - No overly embellished designs.
 - No shorts, athletic / leisure wear, leggings or overalls.
- Footwear
 - Must have a non-slip sole and be clean in appearance and in good condition.
 - House slippers are not permitted.

General Appearance

The ICC is a business environment with frequent visitors. All attire must be neat, clean, free of wrinkles, and in presentable condition.

On weekdays (Monday – Friday, non-company holidays) employees must adhere to the previously outlined dress code.

On weekends (Saturday and Sunday) and company holidays, a GMR branded t-shirt or hoodie may be worn. The other dress code expectations should still be adhered to.

Personal Appearance and Hygiene

- Hair
 - Must always be kept clean and neat.
 - Must be styled in a way that does not interfere with the field of vision or assigned duties.
 - Must not present an unprofessional image. Including, but not limited to:
 - Mohawks and other unusual styles.
 - Neon, glittering or multiple artificial hues visible.
- Facial hair
 - Must be kept trimmed and tidy.
- Fingernails
 - Must be kept neat and clean.
 - Length must not inhibit performance of job duties.
- Jewelry
 - Must portray a professional image.
 - Must not compromise safety.
 - Must not inhibit performance of job duties.
- Tattoos
 - Must be covered if they are perceived as offensive or vulgar.
- Scents
 - All employees should take steps to avoid strong scents in the workplace. Including, but not limited to:
 - Bad breath
 - Body odor
 - Tobacco products
 - Heavy colognes/perfumes/lotions/essential oils
 - Communications staff must comply with field grooming and safety policies if they are to attend any ride-along sessions.

Reasonable Accommodations for Dress and Grooming

The Company is committed to complying with all applicable federal, state, and local laws regarding reasonable accommodations. Employees who require modifications to the dress code due to a sincerely held religious belief, observance, or practice, or because of a disability or medical condition, may request an accommodation.

Examples of accommodations include, but are not limited to:

- Allowing religious attire or grooming practices (e.g., hijabs, turbans, yarmulkes, modest clothing, facial hair).
- Adjusting uniform requirements for medical reasons (e.g., alternative fabrics or styles to prevent irritation or accommodate medical devices).

Employees seeking an accommodation should notify their leadership and Human Resources as soon as possible. The Company will engage in an interactive process to determine a reasonable solution that does not impose an undue hardship on business operations or compromise workplace safety.

Uniform Replacement

Leadership may approve uniform replacements for wear and tear or size changes, with items exchanged on a one-to-one basis. Employees will be responsible for the cost of replacing uniform items due to loss, misuse, abuse or lack of care.

Return of Uniforms

All company-issued clothing remains the property of GMR. Upon the conclusion of employment in the ICC, employees are required to return all company-issued clothing and materials. Failure to do so will result in the cost of these items being deducted from the final paycheck.

Additional Information

If an employee's appearance or grooming does not align with Company standards, they may be required to leave work without pay to address the issue before resuming their duties.

Company-Issued ID Badge

Policy Number: 100-08	Subject: Company-Issued ID Badge
Revision Date: November 11 th , 2025	Effective Date: January 1 st , 2026

I. PURPOSE

To establish procedures outlining the use and possession of company-issued ID badges.

II. POLICY

Badge Possession and Use

- Employees must always display their ID badges inside of GMR facilities.
- Employees must not lend their badge to another individual for any reason.
- If an employee arrives for duty without their badge, they are required to inform the on-duty leader upon arrival to request a replacement or temporary badge.
- Employees are required to swipe their ID badges on any door equipped with a badge reader before entry to ensure that access is properly documented; tailgating is not permitted.
- All visitors, including GMR employees not based at HQ, must register at the front desk. Contact an on-duty leader for assistance.
- Employees must not grant anyone access to GMR property without the approval of leadership.

Missing Badge

Missing ID badges present a security concern and must be reported to the on-duty leader immediately.

- The initial replacement will be provided at no cost to the employee.
- Subsequent replacements may result in a \$15.00 payroll deduction.

Return of Badge

ID badges are to be turned in to the on-duty leader upon separation of employment.

Lockers and Personal Belongings

Policy Number: 100-09	Subject: Lockers and Personal Belongings
Revision Date: November 11 th , 2025	Effective Date: January 1 st , 2026

I. PURPOSE

To establish standards for the property of ICC employees left in lockers or in the common areas of the ICC.

II. POLICY

Lockers

- Are the property of GMR.
- Will be assigned by departmental leadership.
- May have to be shared by employees due to limited availability.
- May only be used by the designated staff member(s).
- May not have anything affixed or attached to either the interior or the exterior.
- May be unlocked or reprogrammed by leadership without employee consent if locker procedures are in question or in the event of an emergency.

Items prohibited from being stored in lockers include, but are not limited to:

- Alcohol
- Illicit Drugs
- Dangerous chemicals
- Flammable or explosive materials
- Perishable items, such as food and beverages
- Unsealed containers holding liquids of any kind
- Weapons of any kind

Improper use of a locker may result in the suspension or termination of locker privileges.

Employees have no reasonable expectation of privacy for items placed in lockers. Leadership may inspect items, and any contents within, at any time based on reasonable suspicion, with or without prior notice.

Personal Belongings

- Must be stored in an employee's assigned locker when off duty.
- May be discarded without notice if left outside of a locker.

GMR is not liable for any personal belongings, including those in lockers. The company will not reimburse employees for discarded items.

Employees are expected to cooperate in any search or inspection of their person or property requested for reasonable suspicion as described in the Company handbook.

Condition and Upkeep of Facilities

Policy Number: 100-10	Subject: Condition and Upkeep of Facilities
Revision Date: N/A	Effective Date: January 1 st , 2026

I. PURPOSE

To establish standards and expectations of workplace cleanliness and facility upkeep.

II. POLICY

Food and Beverage

Employees are prohibited from storing food or beverages in locations other than in the breakroom or in personal lunch bags or coolers. The GMR HQ refrigerator policy should be followed, including attention to the cleaning schedule.

Food and beverages may be consumed at workstations if:

- Work duties are not impacted.
- No residue is left behind.
- Equipment and furniture are not damaged.

Employees are responsible for keeping their work areas clean, including addressing spills, crumbs and overflowing trash. For cleaning supplies or assistance, employees should see an on-duty leader.

Maintenance

Employees should report any malfunctioning, missing, or broken equipment, furniture, etc. to an on-duty leader.

Regular Cleaning

Shift supervisors or their designee will inspect and clean departmental workstations regularly. Personal belongings are not permitted to be stored at any workstation, including in drawers or cabinets. Personal items left outside lockers may be discarded without notice. GMR is not liable for any personal belongings, including those in lockers. The Company will not reimburse employees for damaged, discarded, lost or missing items.

Tobacco and Smoke-Free Workplace

Policy Number: 100-11	Subject: Tobacco and Smoke-Free Workplace
Revision Date: November 19 th , 2025	Effective Date: January 1 st , 2026

I. PURPOSE

To establish a policy regarding tobacco or synthetic product use when reporting for and while on duty in the ICC.

II. POLICY

To ensure a safe, healthy workplace, all tobacco products, including, but not limited to those as defined below, are prohibited in company workspaces, common areas and company vehicles. This policy applies to all employees, clients, contractors and visitors.

- The term “smoking” refers to the burning of all forms of tobacco included in cigarettes, cigars, and pipes.
- The term “smokeless tobacco” refers to non-burning forms of tobacco and synthetics, including chewing tobacco and electronic cigarettes/vapes.

Users should do their best to be free of the scent of tobacco or synthetics on their person when reporting for duty and at the conclusion of each break should they partake during their break.

Smoking and the use of smokeless tobacco products is solely permitted in designated outdoor areas during scheduled break times.

All tobacco products and associated paraphernalia must be stored out of sight while in Company facilities or vehicles.

Break schedules will not be modified to accommodate smoking or tobacco use.

Phoenix: Tobacco and Electronic Cigarettes

Policy Number: N/A	Subject: Phoenix: Tobacco and Electronic Cigarettes
Revision Date: N/A	Effective Date: N/A

I. PURPOSE

The Communication Center policy is to provide a safe and pleasant environment for our customers and employees. With this in mind, the use of tobacco and electronic smoking products are restricted unless otherwise governed by applicable law.

II. POLICY

1. Any type of smoking and the use of smokeless tobacco is only allowed in designated smoking areas during designated break times.
2. Designated smoking area for the Communications Center is the West side of the building at the curb and beyond.
3. No designated area is allowed inside of the Communications Center
4. Within the designated smoking area smokers are to use the disposal method provided for cigarettes. The area is to be kept clean of trash and any smoking material.
5. The sale of tobacco and electronic smoking products on company premises by vending or other means is also prohibited.
6. No smoking within 100 feet of the building.

Any employee who violates this policy may be subject to corrective action, up to and including termination.

III. TOBACCO USE IMPLICATIONS

1. The use of tobacco products will impact the rates you pay for the company provided medical plans.
2. For those interested in quitting, referral programs are available. Please contact the benefits Service Center at 1-888-301-2123 for details, Designated smoking area for the Communications Center is the West side of the building at the curb and beyond.

Inclement Weather

Policy Number: 100-12	Subject: Inclement Weather
Revision Date: November 19 th , 2025	Effective Date: January 1 st , 2026

I. PURPOSE

To establish a procedure to maintain adequate staffing at all times, regardless of weather, and to outline expectations when lodging accommodations are made for inclement conditions.

II. POLICY

GMR requires all essential personnel to report for their scheduled shift regardless of weather conditions.

III. PROCEDURE

GMR's mission statement is to provide care to the world at a moment's notice. Although severe weather or disasters may make commuting to work challenging, employees are required to report for duty unless instructed otherwise.

During periods of inclement weather, employees are advised to allocate sufficient time to arrive at work for the start of their scheduled shift. Employees on duty may be expected to remain until their replacement has arrived.

Employees are responsible for planning for inclement weather. Tardiness or absences will not be excused due to weather events and failure to report for duty when accommodations have been offered will count as an Occurrence of Significant Impact per the Attendance Policy.

The Company will provide reasonable lodging accommodation when deemed necessary. Employees must agree to all stipulated terms prior to accepting the offer of company-sponsored accommodation. Those stipulations are:

- Employees must act professionally as company representatives during GMR-sponsored stays.
- Booking confirmations are sent to the employee's GMR email address.
- A government-issued ID is required for check-in.

- GMR covers accommodation and taxes. Employees must present a credit or debit card at check-in for an incidental hold. Hold amounts and refund timing vary by hotel policy.
- Employees are responsible for any expenses beyond the room rate and taxes, including but not limited to:
 - Additional fees charged if an employee elects to bring their pet(s).
 - Charges for missing items or damaged contents of the room.
 - Room service or incidental fees charged to the room.
- Due to limited hotel availability, it may not be possible to place all staff in the same hotel.
- Transportation between the hotel and office will be the responsibility of each employee.
- Employees are expected to arrive equipped with all necessary items required to stay throughout the entirety of the weather event. These provisions may include, but are not limited to:
 - Food
 - Beverages
 - Toiletries
 - Extra work clothes
 - Outerwear, including appropriate footwear for the expected weather
- Snacks, sandwich-making material, and various microwaveable food options may be available while on duty.
 - Off-duty meals are not provided.
 - Employees should bring their own food to accommodate any personal dietary needs or preferences.

Lactation Accommodation

Policy Number: 100-13	Subject: Lactation Accommodation
Revision Date: N/A	Effective Date: January 1 st , 2026

I. PURPOSE

To establish departmental expectations and procedures for lactation accommodations in alignment with the GMR Employee Handbook.

II. POLICY

This policy ensures compliance with applicable laws and supports employees returning to work after childbirth who require lactation accommodations.

Employees needing lactation accommodations should notify their Leader upon returning to work following childbirth. Upon notification, departmental leadership will meet with the employee to review the request and coordinate appropriate arrangements that balance operational needs and legal requirements.

III. PROCEDURE

Break Time

Employees may take reasonable break time to express milk. When possible, these breaks should coincide with existing break periods. Additional unpaid break time may be arranged and must be accurately recorded using the Company's timekeeping system.

Private Lactation Space

The Company will provide access to a private, non-bathroom space that is shielded from view and free from intrusion. This space will be equipped for milk expression, including seating, electrical outlets, and surfaces as needed.

Milk Storage

Employees may use personal cooler packs or store expressed milk in a designated refrigerator/freezer. All containers must be clearly labeled with the employee's name and date. Employees are responsible for removing containers at the end of each shift. GMR is not liable for items left in shared spaces.

Legal Compliance

In jurisdictions with specific lactation accommodation laws, the Company will comply with all applicable requirements, including providing greater or different benefits than those outlined here.

Respect in the Workplace

The importance of Respect in the Workplace

- Provides a feeling of security at work
- Shows appreciation for patients, coworkers, and leaders
- Maintains efficiency
- Fosters a happy work environment



Cambridge dictionary defines respect as:
“to treat something or someone with kindness and care”.

How to Foster Respect

Avoid gossip	Be mindful of everyone’s feelings	Show appreciation to each other
Greet everyone with kindness	Value diversity and other’s opinions	

The Main Floor

Rules:

1. Use teams when collaborating with coworkers, whenever possible
2. When you do need to talk on the floor, please speak as quiet as possible
3. No horseplay or other noisy activities will be tolerated

Why?

- Helps maintain a professional work environment
- Assists dispatchers and nurse navigating with hearing patient needs, while on the phone
- Assists with keeping patient’s calm when there is minimal background noise

Teams Decorum

1. Nurse Navigation promotes a collaborative work environment when trying to aid our patients or troubleshoot issues we may have run into.
2. This will be a no judgment zone!
3. Feel free to see if others may have handled the situation differently or let us know how you are feeling about a particular situation.
4. This forum is also useful to discuss obstacles you may have experienced.
5. Let’s troubleshoot and work together.

Contracts, Territories, & Licensing

Please see the [Contract Quick Links](#) section for a full list of the territories we serve.

911 Contracts

- Fire Departments handle Emergency Medical Services
- NN is typically contracted through County Fire and Rescue
 - Sometimes as stand-alone solution
 - Sometimes in conjunction with AMR ground transport services
 - AMR offers 911 call center, emergency dispatch, and emergent/non-emergent transport solutions

Licensure

Compact Licenses	Non-Compact License
Compact licensure covers <i>most</i> of our bigger contracts	States not covered • CA, DC, NY, OR, MI

Please provide the following by the end of your first week of employment with GMR:

- Marriage and/or divorce certificates
- How to order official school transcripts (high school diploma needed for LVNs)
- Name and/or gender change documents
- List of schools attended
- We will do fingerprint hard cards in house (must bring a photo ID)

Reference Topics

- [Welcome to Global Medical Resource \(GMR\)](#)
- [Our Vision](#)
- [Our Mission](#)
- [Core Behaviors](#)
- [911 Nurse Navigation](#)



Welcome to Global Medical Resource (GMR)

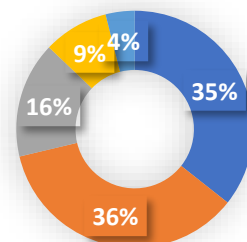
At Global Medical Response (GMR), we foster a culture of service, respect, collaboration, and accountability that guides our decisions and interactions. We believe that our actions, rooted in shared values and behaviors, are what unite us as a company and set us apart in the emergency medical services industry.

As a healthcare organization, that means keeping care at the center of everything we do. It means being responsible stewards of the trust that communities put in us and keeping decisions in the hands of those closest to the patients and community. It means expecting team members to raise their hand and use their voice with the assurance that we hear them, and the understanding that open dialogue is critical to working together to attain our collective goals as a company.

Our culture is the foundation upon which we serve patients, support communities, and build enduring relationships internally and externally.

2024 Survey Results

- Percieved it was a life-threatening emergency (1262 Responses)
- Didn't know how else to get care (1277 Responses)
- Had no other transportation options available (556 Responses)
- Thought treatment would be faster (323 Responses)
- Trouble getting doctor appointments or care elsewhere (125 Responses)



Our Vision

We will provide a healthier tomorrow for millions of people, one patient at a time, by providing access to the right care and serving as a trusted EMS healthcare partner in every community.

Our Mission

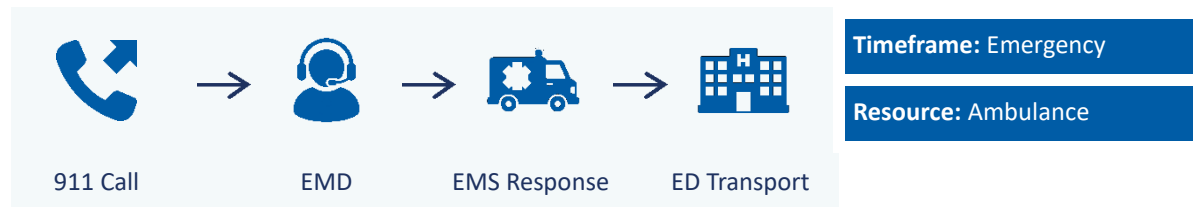
Providing care to the world at a moment's notice

Core Behaviors

GMR Core Behaviors Outline				
KEEP CARE AT THE CENTER We prioritize the care of our patients and our teams when we plan or act. 	RAISE YOUR HAND We ensure that people can feel and safely “raise their hands” about issues, concerns, and opportunities – and it is everyone’s duty to do so. 	SEEK TO UNDERSTAND We treat each other respectfully. We gather information, listen and seek to understand before acting 	FIND A WAY TOGETHER We work together to solve problems and find solutions that support our mission. 	BE ACCOUNTABLE We set clear expectations, systems, and processes and trust those closest to the issue to make good decisions and take accountability. 
For more information, visit the GMR Home Page on Sharepoint .				

911 Nurse Navigation

911 Traditional Model: You Call, We Haul!



This traditional model cannot keep up with modern changes, concerns, or capacity needs.



Consider the modern challenges of maintaining this model. Think about challenges you have faced in your career caused by resource limitations.

- Caller in need contacts 911
- The answering 911 Emergency Call Dispatcher asks Emergency Medical Dispatch questions
- Fire department sends paramedic / EMTs to the scene
- Pt treated in place or transported to the hospital for further eval
 - The individual in need may decline care
- All medical 911 calls treated as emergency w/ ambulance sent to patient
 - Not all ambulances responses are the same!
 - ALS
 - BLS

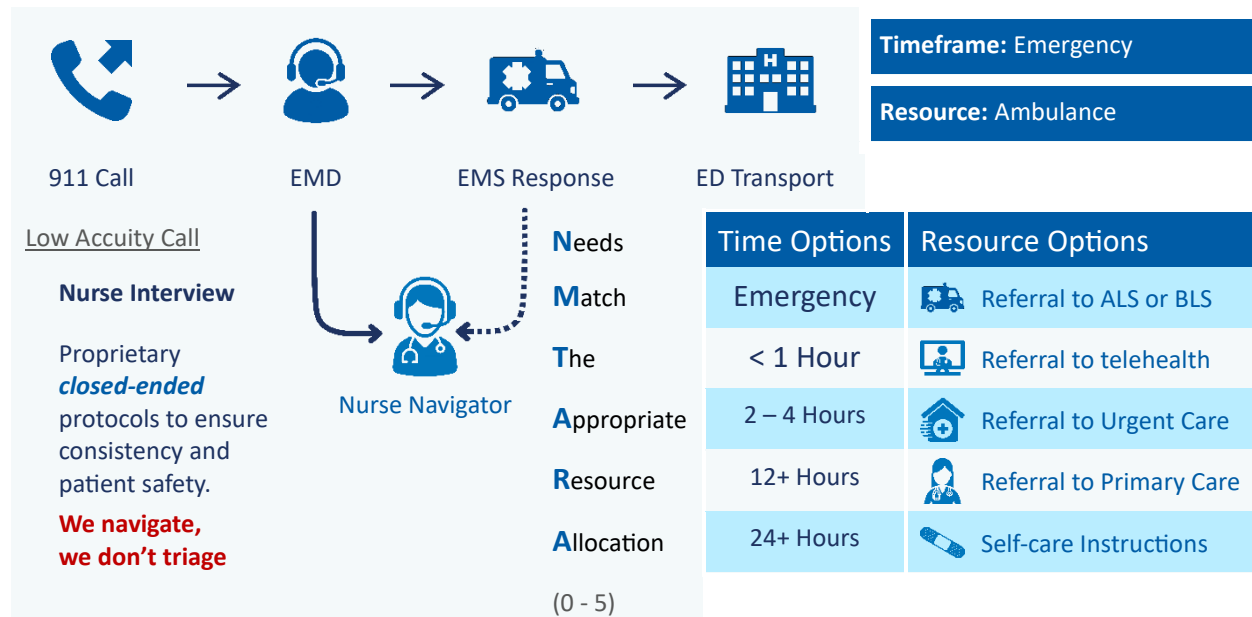
Current Challenges the Traditional Model Faces

- Shrinking workforce: COVID prompted drastic exodus from EMS positions
- Skyrocketing expenses: wages, vehicle maintenance, gas, technology
- Stagnant revenues: Reimbursements have not kept pace with rising costs
- Ever increasing demand from:
 - Growing public
 - Uninsured and under-insured patients
 - Growing medical illiteracy

Why was Nurse Navigation (NN) Initiated?

- 911 NN started as a little project within “Integrated Health”, a GMR department linked to several patient care projects
 - Example: Medically at Home, Envision Physician Services, etc.
- COVID-19 saw NN pursue all kinds of services:
 - Medical screenings
 - Worker’s Complains
 - After-hours triage for MD offices
 - Other similar services
- Most of these ancillary services ended by 2023 coupled by rapid growth in the 911 space
- In 2025, GMR executive leadership refocused GMR from transport to healthcare and positioned 911 NN as the “scaffolding on which EMS 2.0 can be built.”

Our Modern Model



The Ideal Nurse Navigator

- Shows Empathy
- Actively Listens
- Advocates
- Investigates
- Educates



We match the patient’s need to care resources. It’s not just a GMR core value, it’s core to the nursing profession!

Low Acuity Callers

We give low acuity callers treatment options without engaging EMS

Most low acuity callers:

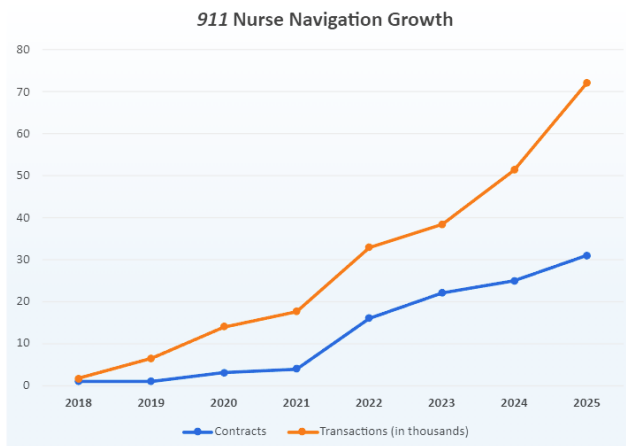
We know:

- 7-8% of 911 EMS calls need time-sensitive, life-saving intervention.
- Conservative estimates show 30-40% of low-acuity, non-emergent calls can be resolved outside an ED: Urgent Care, Virtual Care, BMH services, social services.
- Less conservative estimates have this at 70-90%.

When we fulfill our purpose, secondary benefits include:

- More ambulances available for emergency calls
- Less ER crowding
- Increased patient satisfaction
- Increased EMS provider satisfaction

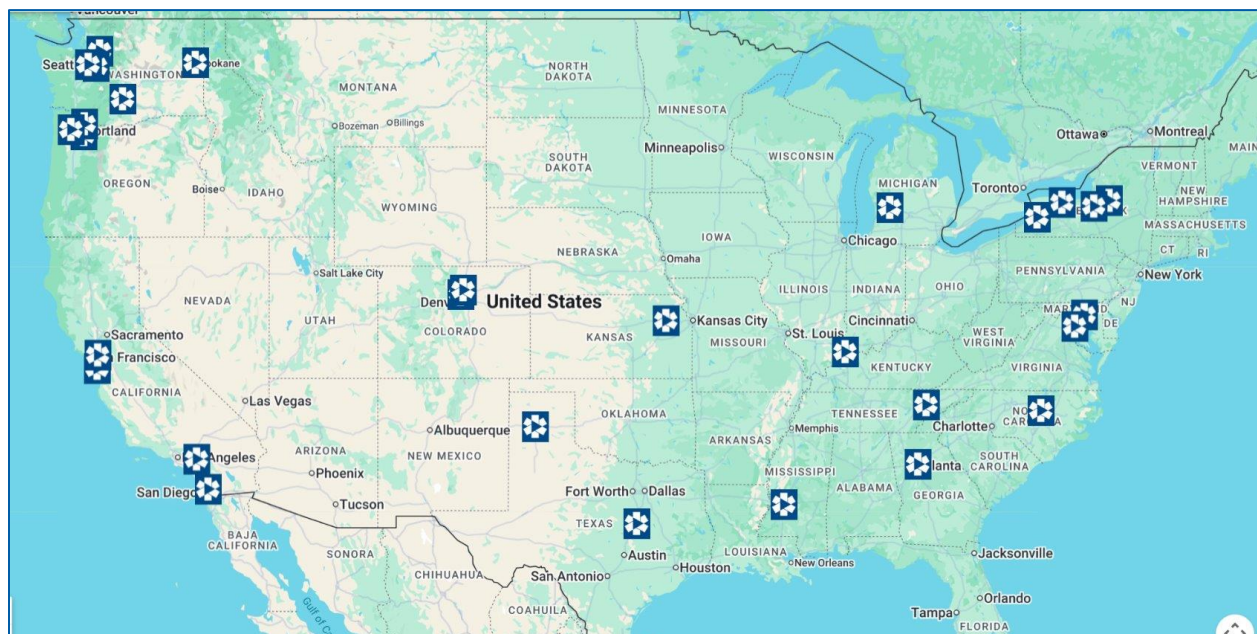
Nurse Navigation Growth



- 5 contracts in implementation
- 10 in negotiation
- 10+ being pursued
- Current contracts looking to increase volume:
 - Expand determinants
 - Start field services
 - Implement CAD-CAD

Contracts By Location

CA	Contra Costa	NY	Oneida County
CA	Riverside	NY	Onondaga County (Syracuse)
CA	Santa Clara	NY	Rochester / Monroe
CO	Boulder	OR	Clackamas
CO	Longmont	OR	Washington County
DC	Fire and Emergency Medical Services	TN	Knox County
GA	DeKalb	TX	Amarillo
IN	Evansville	TX	McLennan County (Waco)
KS	Shawnee County (Topeka)	WA	Clark County (Vancouver)
MO	Baltimore	WA	King County
MI	Kent County	WA	Seattle
MS	Hinds County	WA	Snohomish
NC	Wake County (Raleigh)	WA	Spokane
NY	Erie County	WA	Yakima County



State (Abbreviated) – City	Back to Contract Selection
Special Verbiage / Greeting	

PENDING

Reference Topics

- [Call Types](#)
- [Identifying the Patient's Complaint](#)
- [Leading with Empathy](#)
- [De-Escalation](#)
- [Conversation Guide](#)
- [Potential Call Barriers](#)
- [Frequently Asked Questions \(FAQ\)](#)
- [Dispositions](#)
- [Documentation Do's and Don'ts](#)
- [Follow-Up](#)
- [Quality Assurance](#)
- [Transportation Guide](#)



Call Types

911 Call	FLD (Field Fire)	AMR (Field AMR)
911 Call Reported from EMD Center 	Referral from on-scene Fire Department or EMS staff 	Referral from on-scene AMR Staff 

911 Call

The quality of your interview with the patient will determine:

- If the caller needs an ambulance
 - If EMD needs to use lights / sirens if an ambulance is required
- Is the patient's condition life threatening
 - What care resources match the patient's complaint.

FLD (Field Fire)

- ALS/BLS response requested by 911 dispatch
- Paramedic/EMT on scene evaluated pt to be alternate destination likely
 - Vital signs
 - Medically trained observations
- In-person assessment can reassure that condition is not life-threatening
- Higher chance of successful referral recommendation
- May not have ambulance on scene if pt needs transport (fire truck)

AMR (Field AMR)

- BLS response initially requested by 911 dispatch
- Paramedic/EMT on scene evaluated pt to be alternate destination likely
 - Vital signs
 - Medically trained observations
- In-person assessment can reassure that condition is not life-threatening
- Higher chance of successful referral recommendation
- They are an ambulance on scene if pt needs transport

Identifying the Patient's Complaint

Patient Interview

With telephonic interviewing, our diagnostic abilities are limited:

- We rarely have objective data from diagnostic tools.
- We're asking objective questions about a subjective experience.
- No direct exam / eyes on patient
- No physical exam
- Relies on self-reporting / non-medically trained observations
- Influenced heavily by caller/patient's state of mind
 - Anxiety, Fear, Pain, Anger
- Quality of phone connection.

How to Analyze Pain

Pain is what the patient says it is because:

- It is the individual's experience
- There aren't definitive diagnostic tools to measure it.

This is the #1 rule of Nurse Navigation.



The **symptoms** are what the patient says they are.

Questions

The interview is similar to a pain assessment:

- A patient to quantify their pain – 0-10
- When did it start?
- We ask patient to describe it
 - Area, type, characteristics, & persistence
- We ask patient to evaluate it
 - What makes it better / worse?

Use discretion; we don't have to take *everything* a patient says at face value...



We can use the same types of questions to probe where our eyes, fingers, and stethoscopes can't.

Example: General Communication

Caller



"I have a complaint of..."

GMR Patient Interview Questions



- Have you had these symptoms before?
- Did you seek treatment?
- What did you/they do about it?
- Have you tried that this time?

Example: Immobilization*Caller**"I can't walk."**GMR Patient Interview Questions*

- Are you incapable, or are you being limited by pain?
- Are you too weak, or because you can't move your legs?
- If you need to pee, can you make it to the bathroom?

Example: Immobilization*Caller**"My head, neck, back,
and stomach all hurt!"**GMR Patient Interview Questions*

- Which one started first?
- Which hurts the worst?
- Have you had these symptoms before?
- Did you seek treatment?
- What did you/they do about it?
- Have you tried that this time?

What comorbidities change the complaint?

- Headache with HTN?
- Numbness / tingling with diabetes?
- Nausea / vomiting with a skipped period?

Leading with Empathy

The Cornerstones of Empathy



Active Listening

Listen to understand, not respond. Think about how they are feeling as they describe their situation. For example “that must be frustrating” or “That sounds painful” to acknowledge the caller’s concerns.



Avoid Assumptions

Assumptions lead to unconscious bias which can inhibit equitable and compassionate care. Approach each patient as an individual with unique experiences, beliefs, and concerns.



Offer Comfort

While we cannot hold our patient’s hand, we can still tell them that we are sorry they aren’t feeling well and they we are committed to getting them care quickly.



Avoid Burnout

Take care of your own well-being. Maintaining a self-care routine can motivate you to provide quality patient care. Nurses need to be emotionally and mentally available to advocate for their patients

How Empathy Benefits our Patients

Reduce Patient Distress	Higher Patient Satisfaction	Prevents Emotional Harm	Increased Efficiency
Patients who are anxious or scared believe they are having an emergency. Recognizing this and responding with empathy can greatly reduce their distress and anxiety.	Empathy leads to more positive patient experiences. Patients report higher satisfaction scores when they feel that someone cares.	Empathy helps validate emotions, prevent misunderstandings, and reduce feelings of isolation.	Nurses are more likely to successfully navigate patients to a clinic or other alternate destination

Empathy Phrases

Acknowledgment	Encouragement	Validation
"I can see that this is really distressing for you right now; I'm here to help."	"You're doing great by reaching out; it's a big step to ask for help."	"Your feelings are valid, and it's important that we address your concerns."
"It sounds like you're going through a tough time, and it's completely understandable to feel this way."	"You're taking the right action by calling us; we'll figure this out together."	"It's completely normal to feel anxious in situations like this; let's see how we can help."
"I can see that this is really distressing for you right now; I'm here to help. How long have you been feeling this way?"	"I understand that this situation is very frustrating, and it's okay to feel overwhelmed. Let's talk about what's been happening and see how we can assist you."	"That sounds really difficult; I understand how concerning this must be for you. Can you tell me more about what you're experiencing?"
Clarifying Intentions	Reassurance	Supportive Promises
"My goal is to ensure you get the best care possible; let's talk about what's happening."	"I'm here to listen and support you. We'll work through this together."	"I will do my best to assist you and get you the help you need."
"I want to make sure we understand your situation fully, so we can provide the right support."	"You're not alone in this; I'm here to help you get the care you need."	"Let's take this step by step; I'm here with you throughout this process."
"Right now, you are my priority. Take the time you need."	"You did the right thing in calling us."	"I'm going to talk you through this, but please let me know if you have any questions along the way."

REMEMBER: Your patient may be perceiving a life-threatening emergency, so they are not having a good day. Your job is to assess the situation and provide reassurance and empathy while navigating them to an appropriate care destination.

Your patient is more likely to agree to navigation if they feel heard and cared for.

De-Escalation

Why does it matter to de-escalate?

- Callers who are in crisis cannot think clearly and make decisions even when a nurse is providing a reasonable solution to their problem.
- Prevents unnecessary emergency utilization: Proper education and assessment can avoid overuse of 911 and ER resources.
- The patient's expectations are based on their limited knowledge and understanding of the program.

Core Concepts

- Empathy
- Active Listening
- Validation
- Communicate Clearly
- Set Expectations
- Prioritize Safety

Remain Calm

- Be calm and composed, this isn't about you.
- Use a softer, reassuring tone, avoid raising your voice.
- Avoid arguing or dismissive concerns, promote a safe space.
- Ask open ended questions with slow, deliberate speech.
- Breathe deeply to maintain your own composure.
- Maintain a professional, compassionate demeanor regardless of caller behavior.

Validate what the caller is saying:

- Show genuine interest and focus on the patient's words.
- Maintain a respectful attitude.
- Acknowledge feelings without judgment.
- If you actively listen, you can demonstrate they are understood.
- Listen with the intention of understanding instead of arguing.
- Repeat back what the patient says in a different way.

Get the caller to participate:

- Avoid arguing or dismissing concerns.
- Speak to the patient as you would a good friend or family member.

Redirect focus to something tangible

"Let's take a moment to breathe together."

Use grounding statements

"Let's focus on one step at a time."

Distraction helps to confirm information about their location

Can you tell me what you see around you? What's something you can focus on right now?"

Distraction helps to confirm information about their location

Can you tell me what you see around you? What's something you can focus on right now?"

Set & manage expectations:

- Transparency is key. This includes the scope of Nurse Navigation.
 - Explain the purpose of nurse navigation and what the caller can expect.
- Inform them about available resources and support systems.
- Involve the patient in developing a plan, confirm they are on board.
- Provide education surrounding the plan. Allow patient to write call back number down.

Conversation Guide

This conversation guide training reconceptualizes the call flow of the Nurse Navigator by chunking the call into segments for easy recall. While there are talking point requirements, this conversation guide is not meant to be a script, but a reminder of what to say and when.

Call Overview



Reference Quick Links

	Info & Suggested Scripting	Barriers
1 About Call Handoffs	View	View
2 Greeting the Caller	View	View
3 Verify Patient Details	View	View
4 Clinical Discovery	View	View
5 Clinical Pathway	View	View
6 Care Suggestions	View	View
7 Plan of Care	View	View
8 Arrange Care	View	
9 Partner Contact	View	

For post-call activities, please see the [Frequently Asked Questions \(FAQ\)](#), [Dispositions](#) and [Documentation Do's & Don'ts](#) sections of this reference guide.

About Call Handoffs

Handoff of the patient from the 911 system (911 operator or Field Staff) to 911 Nurse Navigation. Assume the patient is on the line (or on speaker phone), listening to the interaction. This handoff should be short, professional, and instill confidence in the patient and 911 partner that they are in good hands.

- Listen for the whisper
 - It tells you where the call is from and if the contract has CAD-to-CAD
 - Reminds you to use the appropriate greeting for the contract.

Call Handoff Types

911 Dispatcher

1. Incident address
2. Phone number
3. Brief situation

*Accept calls without
excessive delay*

CAD – CAD

1. Incident address

Field

1. Incident address
2. Brief situation
3. Patient vital signs

*FLD=Field Fire
AMR=Field AMR*

Call Handoff Information Exchange

Push

Info given to Nurse Navigation by dispatcher/EMS

- Should include the “expected” info for call type
- May be more than “expected” info – reference number, patient name, etc.
- Document reference numbers when given

Pull

Info requested by Nurse Navigation from dispatcher/EMS

- Only pull “expected data” when it is not being offered by dispatcher/EMS
- Do not pull info outside of the “expected data”
- It is okay to clarify any pushed data



Quick handoffs allow 911 dispatch to resume answering calls while we take care of the patient. It shows we are competent, efficient, and confirms the patient is in good hands.



Call Greetings should match our contracts and include nurse’s identification.

Greeting the Caller

This greeting explains why the caller/patient is talking to a nurse. Include required talking points but make it your own. This statement conveys empathy and explains to the patient why they are speaking with a nurse.

This reduces confusion, increases understanding and buy in, and allows the patient an opportunity to opt out.

Required Introduction Verbiage

"Hi, my name is *[name]*. I'm a nurse on a recorded line."



Use this greeting with every new person you talk to throughout the call.

Set expectations with the required talking points:

- ✓ Reintroduce yourself
- ✓ Inform the patient / caller they are on a recorded line
- ✓ Provide contract name as part of your introduction.
- ✓ Inform them no ambulance is currently on the way.
- ✓ Explain why they are speaking with a nurse.

Phrases to Avoid

- ✗ You don't qualify for an ambulance.
- ✗ "It's going to be a long wait" (or specifying several hours of wait time)
- ✗ "EMS is backed up or no ambulances available in your area"
- ✗ "This is not an emergency"

Example #1: Introductory Verbiage

"Hello caller. my name is [name]. I'm a nurse and this call is being recorded with [contract name] emergency services. I'm sorry you're not feeling well today. At this moment, an ambulance has not been dispatched. I'd like to ask you a few questions to better understand what's going on with you and how we can best help you. In some cases, care options outside the hospital may be a good fit and could help you get the care you need and may save you time and money."

Example #2: Introductory Verbiage

"This is [name]. I am a nurse on a recorded line with [contract name] 911. Your call was sent to me to investigate further and potentially offer you care options that can possibly keep you out of the hospital. I need to ask you some questions to determine what's going on with you today."

Example #3: Introductory Verbiage

"Hello caller. We are speaking on a recorded line. My name is [name] and I'm a nurse with [contract name] emergency services. I hear you aren't feeling well. There is no ambulance on the way right now, but we'll make sure you get the care you need. I do need to ask you some questions to do that."

Example #4: Provide Additional Verbiage

"Based on the answers you've already provided, you qualified to talk to me and I will match the care you receive with your current situation. Right now, there's no ambulance enroute, but I'm happy to get one if that's the best match."

Example #5: Next-Steps Verbiage

"You are eligible to speak with a nurse to assist you with getting care today. I will ask you more questions about what's going on and, if needed, we'll be happy to send you an ambulance."

Verify Patient Details

Verify the patients identify by asking 3 data points:

- First and Last Name
- Date of birth
- Phone number



Verify the spelling of the patient's name to confirm accuracy of records.

Other data points to collect are **gender** and **insurance type**:

- Age and gender are important and guide our line of inquiry and ensure that we probe appropriately to get them the care they need.
- Insurance type should be collected for all patients to match with the best care options, allow 911 Nurse Navigation to trend resource usage, and explore service gaps with our contracts / partners.

Example #6: Patient Verification Verbiage

"For your security, please verify your full name, date of birth, and phone number. Is that the traditional spelling?"

Example #7: Patient Verification Verbiage

"To comply with our patient privacy standards, can you please verify your first and last name to proceed? And your date of birth? How about your phone number?"

Example #8: Patient Verification Verbiage

"To avoid unnecessary questions and make sure our questions align with you medically, can you please confirm your gender?"

Example #9: Patient Verification Verbiage

"My system adds/removes questions based on age and gender. Can you please confirm your sex for me?"

Clinical Discovery

Ask probing questions to understand their chief complaint, allowing you to match the patient with the most appropriate protocol, resulting in a more accurate NMTARA level.

- Your goal is to gather enough relevant information to be confident in the pathway.
- Primary source-data from the patient is priority.
 - Always ask to speak with the patient unless inappropriate (i.e. age, developmental level, aphasic, etc).

Assess patient responses using your judgement and immediately escalate to an ALS response if needed.

Use **OLDCARTS** or another mnemonic to guide your probing:

O	Onset
L	Location
D	Duration
C	Characteristics
A	Aggravating / Alleviating factors
R	Radiation
T	Timing
S	Severity

Example #10: Clinical Discovery Verbiage

"Tell me more about your [condition]. When did it start and has it been treated?"

Example #11: Defining Pain

"Describe your pain. Does it feel sharp and stabby or dull and achy?"

Example #12: Pain Intensity

"How would you describe your pain? Is it tolerable, annoying, distracting, unbearable?"

Example #13: Pain Intensity

"How would you rate your pain on a scale of 0-10"

Example #14: Pain Intensity

"How does your [condition] impact your daily activities like eating, sleeping, and toileting?"

Clinical Pathway

Follow the clinical pathway to completion. These are: MD approved questions. Clinical protocols are **NOT** optional. Think of them as your failsafe.

Ask all questions to ensure you reach an accurate **NMTARA** level. Nurses can always escalate care with a reasonable concern, but **NMTARA** can't be downgraded.

- If you believe the NMTARA level is falsely high, report it so we can track and work on system solutions.



Clinical protocols are followed to the end; All questions asked or confirmed. All steps completed.

Verbiage

Keep the caller / patient talking and informed of next steps, once determined.

Care Suggestions

Consider the patient's circumstances and resource availability. A hierarchy of choices will be presented to you.

- Select the closest match to the **NMTARA** level (i.e., REF or VC for NMTARA 1-5)
- For your consideration:
 - Does insurance play a factor?
 - Clinics, specialty resources, Kaiser, etc...?
 - Will they potentially need imaging or lab services?
 - What care resources are available for their area?
 - Does the patient have mobility restrictions?
 - Can the patient be safely transported in a rideshare?



If any of these options limit your choices, document it.

Example #15: Urgent Care Referral

"Based on your answers, I think going to a nearby urgent care clinic will be best for what's going on with you today. Do you need me to arrange a ride for you?"

Example #16: Virtual Care Referral

"Based on your answers and your insurance, I recommend seeing a virtual urgent care provider for your issue. You'll speak with the intake coordinator before you are connected to the provider."

Example #16: Virtual Care Provider Explanation

*"We have a **Doctor / Provider** here you can speak with so you don't have to leave the comfort of your own home. We have ER and Urgent Care doctors that can prescribe medications, if needed, and send them to whichever pharmacy you choose. We would also provide the ride there, and back, when the medication is ready for pickup."*

Example #17: EMS Transport

"Because you cannot safely enter or operate a vehicle at this time, I will arrange an ambulance to take you to the ER."

Plan of Care

The patient must be informed of the plan of care. This includes verbal consent to the plan. Summarize the call and actions taken to ensure the patient has a clear understanding of next steps:

- Where they are going?
 - Clinic or hospital name and address.
- How they will get there?
 - Rideshare, ambulance, or private vehicle.
- What should they take with them?
 - Photo ID and insurance card; the clinic may also collect a co-pay.
- Who to call when the plan breaks or they need a ride home?
 - Provide the patient call back number and explain what it's for.
- Home care instructions?
 - Documenting only what was verbalized.
- Will there be a follow-up call or communications?
 - Follow up to get their thoughts and feedback regarding the care they received.



The patient is aware of the specifics regarding the plan of care and they know who to call if the plan breaks.

Example #18: Virtual Care Plan

"Based on your symptoms, we determined that virtual care is the best care option for your sore throat. I am going to conference in a care coordinator. They will ask you to verify your information, like I did, and collect your insurance information as well - similar to an in-person clinic visit. After that, they will connect you with a provider. We want to make sure we met your expectations. Keep an eye out for a call from us in the next couple of days; we'd love to hear your feedback."

Example #19: Transportation

"I have arranged a ride to pick you up in 5 minutes to take you to [clinic name] located at [clinic location]. [Driver name] will pick you up in a blue Honda. I've told them you are wearing a red jacket and blue jeans. Please take down this phone number so you can give us a call when you are done at [clinic name] and we will get you back home. We will follow up with you tomorrow to conduct a survey about your experience with 911 Nurse Navigation."

Arrange Care

The nurse arranges care agreed upon by the patient. Ensure the patient understands the plan of care and who to contact if the plan breaks. The patient has the chance to ask questions and decline before care is arranged.

Examples of Care Options

- Select an open clinic and verifying they accept the patient's insurance
- Arrange transport to a community care resource
- Conference RelyMD for a virtual care visit
- Correctly activate an ambulance response based on contract directions
- Inform the patient of the plan
 - Where they are going
 - What they need to take with them
 - How they will get there
 - What is expected of them
 - Who to contact if the plan breaks and/or they need a ride home

Example #20: Ride to Clinic Next Steps

"I am arranging for you to be seen at the [clinic name] and I will set up a ride to take you there and back home. Please start getting ready while I set that up for you. I will place you on a brief hold while I do that."

Example #21: Virtual Care Next Steps

"Let's recap your call to 911 Nurse Navigation, your initial complaint was [xyz] and based on the assessment, we agreed that a virtual care visit is our next step. Please take down this phone number in case you can't complete your virtual care visit or if you have any issues. You may hear silence followed by a brief hold as I get that set up for you. You will answer some questions with the care coordinator before speaking with the provider."

Partner Contact

When calling any partner (dispatch, VC, Crisis Connections, etc.) always:

- Identify yourself
- Who you work for
- Why you are calling

Example #22: Ambulance Request

“This is [name] with 911 Nurse Navigation. I’m calling for an ambulance for [address] due to [patient condition, refusal of resources, limited mobility, etc]”.

Example #23: Virtual Care Request

“This is [name] with 911 Nurse Navigation. I have a patient in need of virtual urgent care.”

Virtual Care-RelyMD Process

After checking the VC exclusion list:

1. Explain the RelyMD process to the patient (set expectations)
2. Follow Corti to arrange care.

Potential Call Barriers

Call Handoff



The dispatcher does not have an exact address, only cross streets.

Enter the cross streets into Google maps (i.e. “intersection of 7th and Main (city/state)”). Ask the patient to describe what they see around them and correlate with information from maps.



The nurse is having trouble geolocating the address in Logis.

Don't geolocate here. Wait until you are on with the patient to clarify address and geolocate.



The patient is yelling / belligerent during the handoff.

Wait until the patient stops talking and offer empathy, provide context, and preview next steps as needed.



The nurse continues to ask for excessive information from dispatch

Only ask clarifying questions. All other information can be confirmed with the patient / caller, including geolocation for addresses

Greeting the Caller



The caller / patient interrupts and demands an ambulance immediately

Wait until the patient stops talking and offer empathy and a why

- “I’m sorry you’re not feeling well today. I’m a nurse and I am here to get you the help you need today.”



The caller / patient does not speak English

Conference in the language line services; see Corti for instructions



The patient refuses to speak on a recorded line

Explain that we record for quality assurance. if they still refuse, end the call and send it back for BLS.

- “I’m sorry. I cannot proceed with this call. I will have an ambulance sent to your location.

Patient Verification



The patient refuses to verify demographic information

Take what information you can and explain the purpose of your request.



The patient identifies as non-binary

Respectfully explain that some questions are included or excluded based on biological sex so that we can best match them to the proper care.



The patient has a 911 only phone

Ask the patient / caller to confirm if it's 911 only. If so, put '911 only' in the number field. If the patient/caller has no phone, put 'no phone' or 'unknown' in the number field

Clinical Discovery



The patient reports red-flag symptoms

Based on your nursing judgement, **immediately** escalate care to an ALS response, if needed, in the event of:

- **New** onset of weakness
- Persistent or worsening pain that interferes with activity
- New onset of confusion or cognitive issues
- Continuous vomiting/diarrhea
 - Especially in the very young and very old
- Difficulty breathing
 - With or without blue lips, mouth, or tongue
- Chest pain/pressure that radiates up to jaw or down arm



The caller is providing answers for patient without consult

Ask to speak directly with the patient if possible

Clinical Pathway



The patient's answer indicates they don't understand the question

- If they don't understand the question as written, rephrase to ask the spirit of the question
- 'And' and 'Or' are very important aspects of the question and should not be omitted
 - Example: (Age 50+) Is there a tearing or ripping pain, or is the pain rapidly getting worse?
 - Example: Are (you/they) having persistent nausea and vomiting?



The patient gives vague answers

- Ask the patient to clarify their answer if needed (i.e., "When you say sometimes, does that mean right now?")
- Ask for a concrete 'Yes' or 'No' answer
- Ensure that your click matches the patient's answer



The patient gives vague answers

- System glitches
- Refresh system [Log out and log back in]
- Use Briggs book if Corti is not functional
 - Need backup for contract routing
- Report pathway issues to Nathan-if questions don't make sense or unable to move forward.

Care Suggestions



The clinic is closed or doesn't take the patient's insurance

- Place the caller on hold and call the clinic to verify their hours and accepted insurance



There are no providers available

Pivot and offer the patient a ride to a clinic



The patient refuses community care resources and demands an ambulance

De-escalate and explain that a clinic or virtual care visit is often faster and more cost effective than an ED visit.

- If they still decline, document refusal and which resources offered.

Plan of Care



The patient has no time to note or save the patient callback number before getting into the rideshare

- Provide the information before requesting the ride so the patient has time to take down the number and fully grasp the plan.
 - Nurse can also schedule the ride to give patient time to get ready (as NMTARA level allows)
- If sending a patient to virtual care, go over the plan with them before conferencing in the care coordinator. Provide the callback number and advise them of a follow-up [survey](#) via next business day phone call.

Frequently Asked Questions (FAQ)

Question	How do we process calls from minors?
Answer	If a minor (younger than 18 years old) calls with no adult present, we will send back to EMS to dispatch an ambulance.
Question	How do I decide between an ALS or BLS response?
Answer	Based on the nurse's identification and evaluation of red flag symptoms and the clinical protocol's care suggestions • BLS Response: Non-Emergent Response ➤ At the normal speed of traffic ➤ Units could be diverted to an emergency call nearby • BLS Response Consideration: ➤ The patient requires services only a hospital can provide ➤ The patient is unable to bear weight, walk, uses a wheelchair, or is bed-bound ➤ Recommended mode of transportation by a healthcare provider • ALS Response: Emergent Response ➤ Lights and sirens ➤ Full speed ahead (goes through red lights) ➤ Won't be diverted • ALS Response Consideration: ➤ The patient requires advanced medical care such as: ○ cardiac monitoring ○ intubation ○ medication administration ○ IV access ○ Life-threatening emergencies like: ○ respiratory distress ○ MI related chest pain ○ stroke-like symptoms

Question What if Transport.net doesn't work for me?

Answer Refer to your NMTARA level guidelines. The timer resets after 4 min of no response, up to 12 min max. Call for a BLS response if no response after 12 min. If pickup time is out of range, be sure to select this in Corti.

Question Is there a list of call scenarios / types we should receive?

Answer Each contract has a list of determinants that tells them which calls are eligible for 911 Nurse Navigation. This list is not for us to screen which calls we accept and assess.

- For more information, please see the [Contracts Quick Links](#) section of this reference guide.

Dispositions

Virtual Care (VC) / Telehealth

Partner: RelyMD

NMTARA 0-5

Clinical Staff	Emergency Department Boarded Physicians as well as other providers like PAs and NPs.
Availability	Available per provider's self-schedule: "Uber"-style model • <i>Check availability before offering!</i>
Insurance	NOT insurance dependent
Age	>2 years old
Platform	Prefer smart-device, but not necessary to connect on scene.

Calls Suited for Virtual Care

Minor Motor Vehicle Crash

- No complaints or minor musculoskeletal complaint
- Low speed, parking lot fender-bender, or rear-end at traffic light
- Ambulatory on scene
- **NOT** for major MVAs on roads that exceed speeds of 45mph

Flu-Like Illness or Common Cold

- Minor upper-respiratory complaints
- Fever without malaise, chills, aches
- *Patient must **NOT** meet sepsis screen as per our usual sepsis protocol*

Cough

- With or without **MILD** shortness of breath
- No hypoxia or respiratory distress

Mild Dehydration

- Does patient have access to fluids?

Animal or Insect Bite

- Minor injury that may / may not need stitches (if dog or cat bite)
- **NO involvement of the joint / tendon**
- **Transport to ED, if possible if patient needs rabies prophylaxis, if bitten by an unknown animal or bitten while in LE custody.**

Wheezing

- History of asthma or COPD
- Improved after one Neb
- No hypoxia or respiratory distress

Ear, Nose, or Throat Complaints

- Conjunctivitis
- Ear Pain
- Minor epistaxis (not on blood thinners)
- Sore mouth or throat

Nausea, Vomiting, or Diarrhea

- Minor cramping
- Must be able to ambulate
- Must be likely to tolerate PO intake
- **NOT orthostatic**
- **WITHOUT severe pain or tenderness**
- **MUST be limited to 1-2 days**

Musculoskeletal Injuries

- Minor lacerations or deformities that may / may not need X-Rays or stitches
- Patient must be able to bear weight and use the extremity
- **ANY bleeding must be controlled (no tourniquet)**

Minor Burns

- Sunburn or first / second degree burns with minimal BSA involvement
- **NO face, fingers, toes, genitalia**
- **NO circumferential burns**

Minor Burns

- Sunburn or first / second degree burns with minimal BSA involvement
- **NO** face, fingers, toes, genitalia

Minor Ground-Level Falls

- Follow the Falls Protocol for DMHC / Event patients
- Cervical spine can be cleared via usual EMS protocol
- **NO** change in ambulatory status
- **NO** syncope
- **NO** Loss of Consciousness (LOC)
- **NO** intoxications
- **NO** obvious significant injury

Low Back Pain

- Acute or subacute exacerbation
- Patient is ambulatory
- **NO** extremity weakness
- **NO** difficulty with bladder or bowel function
- *Chronic back pain is not appropriate for the urgent care environment*

Mild Allergic Reaction

- **No** Epi before arrival and no Epi from responders

Rash or mild cellulitis

- Present on extremity without fever

Dental Pain

- **NO** telemedicine or Urgent Care if patient is requesting opioids. There should not be an expectation that Urgent Care will prescribe opioids, regardless of complaint.

Medication Questions or Refill Requests

- Telemedicine for patients who have isolated medication questions
- **NO** controlled substance refills

Coverage Check

- Make sure you have a provider available in the state your patient is currently in.
- ER means there's an ER doc available for all your NMTARA 0 needs.
- UC means there's an urgent care provider on for all your NMTARA 1-5 patients.

Coverage Requests

Report a Coverage Gap

Use this form to report any instances when no providers are available for telehealth services in a state.

State

(CA) California

Service level

☐ Virtual ER
 ☐ Virtual Urgent Care

Coverage gap occurred

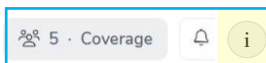
10/03/2024 11:40 AM

Close

Submit

State Coverage			
State	ER	UC	
California	4	6	Start
Colorado	4	5	Start
District of Columbia	1	2	Start
Florida	4	6	Start
Georgia	5	6	Start
Indiana	3	4	Start
Kansas	3	3	Start
Maryland	3	5	Start
New York	6	8	Start
North Carolina	4	5	Start
Oregon	1	1	Start
Tennessee	5	6	Start
Texas	4	4	Start
Washington	2	2	Start

- If you're missing a provider for your patient, you can click the Report button, to help track the specific gap in coverage.
- You can also ask a care coordinator in the chat if a provider available in the state your patient is in.
- Check this [Info](#) icon if you are on the fence about VC. You can get to this list by clicking the info icon to the right of coverage:



Virtual Care Exclusion Criteria

General Exclusions

Age Criteria Exceptions

- All patients under two years old
- Patients 17 or younger without parental / guardian consent

Age Criteria Exceptions

- Patient is non-ambulatory (cannot walk) without available or reasonable assistance from transportation

Medication Management

- Patients likely to require scheduled narcotics or controlled substance refills

Advanced Life Support (ALS) Management

- Any condition clearly requiring immediate ALS intervention

Neurological Symptom Exceptions

Sudden Onset or Severe Headache

- Example: “Worst headache of my life” within the last four hours

Sudden Onset Sustained Weakness, Numbness, or Paralysis

- Especially unilateral within the last 4 hours

Prescreen Questions

Before you enroll this patient for a virtual care visit, please review the following information:

1. If the patient is a minor (< 18 y.o.):
 - RelyMD does not see patient < 2 years of age.
 - Patients age 2-17 need a parent / guardian present to consent for VC treatment.
2. RelyMD does not prescribe scheduled medications.
3. RelyMD is not an appropriate solution for patients with:
 - ALS symptoms
 - Active suicidal ideation or behaviors
 - Active homicidal ideation or behaviors
 - Active psychosis or severe agitation

Virtual Care Teams confirmation

Inform the patient to start their virtual visit, there will be an intake process very similar to checking in at the doctor or ER. Advise the pt that if there is a disconnect during transfer, they should turn off spam blockers and expect a call from an 855 number to complete this intake.

At this time, inform the patient that you will follow up with them if they require further care and if not, they will receive a follow up call to

Then, put the following in the [NN/Care Coordinator](#) chat:

Pt Name, DOB, Address, Active callback number, U.S. State pt is calling from.

Virtual Care Teams confirmation

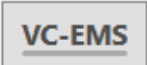
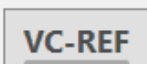
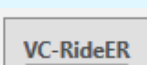
1. Keep the patient on hold to start the transfer.
2. Dial 855-879-4332
 - Option 9, 1, 1.
3. Once the Care Coordinator answers confirm the information sent in the chat and verbally give pt age, gender and situation.
4. Bring patient on the line and confirm everyone is on the call before disconnecting.

Track the Consultation

Monitor the RelyMD chat for updates from the provider. In the event the provider does not send a message, be sure to check the chart and follow through to make sure you do not miss further necessary care.

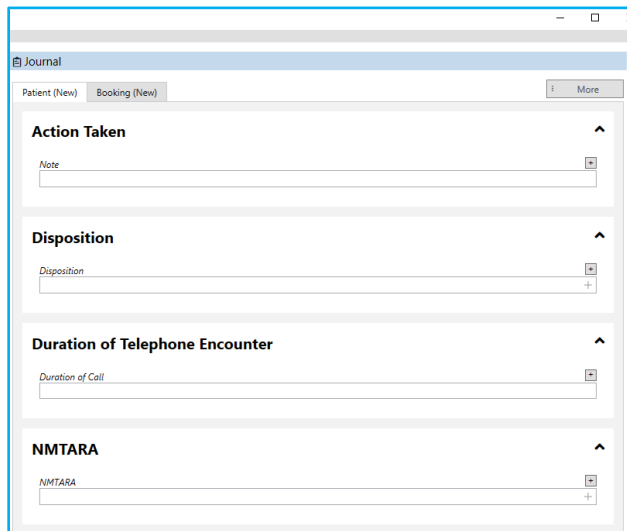
Follow-Through

- If your patient has been seen by the RelyMD provider and they recommend in-person evaluation, follow the steps below:
 - Copy the incident (with comments)
 - Update the Response Plan / Status on the new incident

Selection	Purpose
	The provider recommends they go to the ER via ambulance (ALS or BLS)
	The provider recommends they go to an urgent care clinic
	The provider recommends they go to the ER via rideshare

Journal

- Click the plus sign to add a note that includes the provider's recommendation and the action taken by the nurse.
 - Example: "Nurse Navigation assist to nearest ED via EMS". Patient notified of the provider's recommendation that he/she go to the ER via ambulance. Dispatch called for BLS.
- Click the plus sign to update the disposition to the recommended care option.



Documentation Do's & Don'ts

Someone reading your after-call note should understand clearly what happened without listening to the call.

Documentation:

- Tells the story of the patient encounter
- Provides follow up information
- Demonstrates that we did good work
- Stands as a legal record of patient care.

With that, keep in mind that your documentation should not be unnecessarily lengthy to the point of inefficiency.

- Keep it Short & Simple (KISS)



Deviations from protocol
are documented

SBARF

S	Situation	current situation; chief complaint
B	Background	Relevant history
A	Assessment	Assessment including vitals and Corti summarization (pathway, trigger question, NMTARA level, and Corti session ID)
R	Recommendation	Care options offered/declined/accepted; with deviation documented where applicable
F	Follow through	Actions taken by nurse to facilitate care

If the After Call Work (ACW) timer runs out before you finish documenting and another call comes in:

1. Let the Call Roll.
2. Put yourself into Extended After Call Work (EACW)
3. Finish your documentation.
4. Going forward always note your ACW timer.

Guidelines

✓ DOs

- Accurate spelling of patient's full name and physical address
- Appropriate capitalization and inclusion of apartment / building / unit numbers
- Be concise and clear in your communication
- Enter patient information in the appropriate fields
- Verify who the caller (name) is and their relationship to the patient
- The patient's phone number may be different from the caller's phone number.
 - Do not enter the EMT / Medic's number here.
- Call tags are **NOT** optional! You must select one of three:
 - 911 call taker
 - Field Fire
 - Field AMR

✗ DON'Ts

- Chart opinion-based notes. Only document facts.
- Copy and paste the unedited Corti AI summary.
- Submit journal without checking spelling /grammar
- Document what isn't reflected in the Corti session

Documentation Layout

The screenshot shows a software interface for documenting an incident. At the top, it says 'Workset #1509116 / Incident #'. Below this are tabs for 'Encounter' and 'Home Visit'. To the right are buttons for 'Journals', 'Triage Guide', 'ProQA', and 'PrEval'. The main section is divided into two columns: 'Patients' and 'Customer'. The 'Patients' column has a search bar (A), a patient search field, and an insurance status message. The 'Customer' column has fields for 'Caller' (B), 'Applicant' (C), 'Phone' (D), and 'Incoming Line'. Below these is a 'From' section with an address field (E), 'Apt./Unit' (Floor, Door, Apt.), and a 'Note' field. To the right of the address field is a 'Use patient address' button and a 'No items' dropdown. Below the 'From' section is a 'Booking' section with fields for 'Cause', 'Criteria', 'Tags' (H), and 'Comment'. The 'Tags' field has a dropdown menu with options: '[Call Taker] Call Taker', '[Field AMR] Field AMR', and '[Field Fire] Field Fire'. To the right of the 'Booking' section is a 'Response Plan / Status' section with a 'Contract' field (F) and a 'Ref.' field (G). The 'Ref.' field has a dropdown menu with options: 'None', 'Field Fire', 'Field AMR', and 'Call Taker'.

- A. Click for Patient Info Screen.
 - Enter the patient's information and ensure all required fields are completed.
- B. Enter Contract Name.
- C. Enter the caller's name and relationship to the patient.
- D. Enter the caller's phone number.
 - The caller's phone number may be different from the patient's phone number.
 - Do not enter the EMT / Medic's number in this section.
- E. Enter the patient's address details.
- F. Select insurance type.
- G. Input the Reference / Event Number.
- H. Call tags are NOT optional! You must select one of three:
 - 911 call taker
 - Field Fire
 - Field AMR

The screenshot shows a software interface with a header bar labeled 'Units / Patients' and a sub-header 'Show cancelled: (0)'. Below this is a 'Patients' section containing a single entry: 'Patient, Fake (20636, 25)' with ID '20636'. A 'Planned Destination' section follows, featuring a 'Delivery:' text input field and an 'Edit' button. At the bottom, there is a 'More' button, a blue arrow pointing right, a yellow button labeled 'Being Evaluated', and a row of buttons: '+', 'New', 'Cancel', and 'Save'.

After you open your CAD-to-CAD call on the right side, click the **Being Evaluated** button so that your team will know you are working on the call.

Patient Overview Window

The 'Patient Overview' window is titled 'New Patient (On new incident)' and shows the 'Owner: EvHealth'. It contains several input fields: 'First Name', 'Last name', 'Phone' (with a phone icon), 'Date of Birth' (with a calendar icon), 'Gender' (a dropdown menu currently showing 'Unknown'), 'Insurance Name' (a dropdown menu), and 'Comment'. Below these are 'Requirements' (a dropdown menu showing 'None'), 'Preferred Crew Members', and 'Unwanted Crew Members' (all with text input fields).

Required entry fields are:

- First Name
- Last Name
- Phone Number (use dashes)
- Date of Birth
- Gender
- Address
 - Include apt. / building / unit number

The 'Addresses' window features a 'New' button at the top right. Below it is a large text input field for the 'Address'. A section on the left contains 'Address *' with a search icon and 'Apt./Unit' with a dropdown menu. To the right of this section are buttons for 'View on Map', 'Move on Map', and 'Keep coordinates' (which is unchecked). Below these are buttons for 'Set as Primary Address' and 'Delete'. At the bottom right are 'OK' and 'Cancel' buttons.

Workset #1509116 / Incident #

Encounter Home Visit Journals Triage Guide ProQA PreEval

Patients 1 **Customer**

Search:

Patient: Patient, Ima

Insurance: No insurance information was found.

Caller:

Applicant: Phone:

Incoming Line: Called From:

From

Use patient address

Address:

Apt./Unit: / /

Note:

Booking

Cause: Hold:

Criteria: Ref.:

Tags:

Comment:

Response Plan / Status

Contract:

- 911 NC, Wake County - Commercial
- 911 NC, Wake County - Medicaid
- 911 NC, Wake County - Medicare
- 911 NC, Wake County - Other
- 911 NC, Wake County - Uninsured

ALS **BLS**

VC **NN-RideER**

J REF **Other**

RSC **VC-EMS**

VC-RideER **VC-REF**

Units / Patients Show cancelled: (0) ☐

Patients

Patient, Fake (20636, 25)
20636

To

Delivery:

More

Queue Times

Clinic	Time	Action
Legacy SoHealth UC - Happy Valley, 9361 SE E...	4:00 PM	More
PQH/C, Clackamas Mental Health Center, 11211...	4:00 PM	More
Providence UC - Gateway, 1321 NE 99th Ave, 97...	4:00 PM	More
Providence UC - Reed - Providence Urgent Care...	4:00 PM	More
Adventist Health UC - Sandy, 17005 Ruben L...	4:00 PM	More

+ New **Cancel** **Save**

- I. Select the contract type from the Response Plan / Status menu on the right.
- J. For a clinic referral, select REF from the Response Plan / Status menu.
- K. From the list of clinics, select the one with the closest time.
- L. If the name and address of the desired clinic are not listed in the Queue Times window, enter them manually into the Delivery field.

Editing the Corti Summary

- Add patient info
 - Age; sex; chief complaint
- Reported symptoms and summary of probing questions
- Relevant past medical history and current meds
- NMTARA level
- Recommended disposition and patient's acceptance of plan
- Any education provided to the patient
- Corti Session ID and date/time info

Follow-Up

Circle back to document patient outcomes, if known.

Add a note, **DO NOT** edit an existing note.

- Ride completed
 - Ride home requested / completed
 - Virtual Care follow up
 - TIP, VC > REF, VC > EMS
- Include the clinical disposition
 - What is the consultation outcome?
- **Do NOT include the provider's name in the note**
- Patient needed ride to a different clinic



Ride completion should be documented in a timely manner to confirm completion of transportation and ensure the patient requires no further care from us.

Quality Assurance

Guiding Scope & Principles

- Observers of 911 NN's efforts to ensure safe and appropriate patient care
- Use objective metrics based on stated standards and processes
- Every call reviewed is an opportunity to reinforce what we're doing well, move us closer to a unified patient experience, and identify areas for NN improvement

What We Do

Routine Evaluations	Each nurse is scored twice per month
Call Investigations	Both Internal & External
Special Assignments	Analysis of call types for better understanding, workflow improvement, and systems checks.
Reporting	Research and report trends and findings to the proper stakeholder(s)

Reporting

- Calls within normal expectations will be discussed at scheduled 1:1s
 - **Goal:** same-day visibility on NN "report card"
- Flagged calls allow for timely guidance supporting continual improvement
 - If you report to a Supervisor, reports go to Supervisor and Manager
 - If you report to a Manager, reports go to Manager and Director

QA Rubric Goals & Development

	Developed from the Conversation Guide		Designed to minimize subjectivity and eliminate bias
	Evaluates adherence to workflows		Call Handling & Documentation are both graded

Call Handling

Raise your Hand – 3 pts

- ☐ The nurse used the appropriate greeting for the call. (3 pts)

Keep Care at the Center – 35 pts

- ☐ The nurse informed the patient/caller they were on a recorded line. (5 pts)
- ☐ The nurse appropriately verbalized that there was no ambulance on the way / why the call was routed to a nurse. (5 pts)
- ☐ The nurse verified HIPP data of the patient (name, date, and phone). (10 pts)
- ☐ The nurse verified insurance with the patient/caller. (5 pts)
- ☐ The nurse verified the incident address and disconnect phone number with the patient/caller. (10 pts)

Seek to Understand – 28 pts

- ☐ The nurse spoke with the patient / asked to speak with the patient. (8 pts)
- ☐ The nurse probed chief complaint and red flag symptoms. (10 pts)
- ☐ The nurse chose and accurately followed an appropriate triage pathway. (10 pts)

Find a Way Together – 24 pts

- ☐ Care resources were presented one at a time. (3 pts)
- ☐ Care recommendations were appropriate given NMTARA and known barriers. (8 pts)
- ☐ Plan of Care was outlined with the patient. (8 pts)
 - Where they're getting seen
 - How they're getting there
 - What to take with
 - Who to call for next steps or breaks in the plan
- ☐ The patient agreed/verbalized understanding of the PoC. (5 pts)

Be Accountable – 10 pts

- ☐ Care arrangements initiated per disposition specifications (10 pts)
 - **Ambulance** – responsible party was informed per contract instructions
 - **Private transport** – destination selected in software and transport finalized
 - **Telephonic** – followed instructions to successful warm handoff

Kudos: Did the nurse do something exceptionally well, or go above and beyond on this call?

Documentation

Customer Field – 16 pts

- ☐ Caller (911 Contract) (6 pts)
- ☐ Phone (Disconnect callback number) (6 pts)
- ☐ Applicant (Disconnect phone owner, with relationship if not patient) (4 pts)

Patient Info Field – 24 pts

- ☐ First and Last Name (spelling verified) (8 pts)
- ☐ Phone (verified as pt's) (6 pts)
- ☐ DOB (8 pts)
- ☐ Gender (2 pts)

From Field – 9 pts

- ☐ Address (8 pts)
- ☐ Additional Data (if needed) (1 pts)

Booking Field – 9 pts

- ☐ Cause (triage pathway) (6 pts)
- ☐ Tags (source of call) (2 pts)
- ☐ Referral (if given) (1 pts)

Response Plan Field – 16 pts

- ☐ Contract (insurance) (4 pts)
- ☐ Disposition (8 pts)
- ☐ Resource selected (if applicable) (4 pts)

Journal Screen – 28 pts

- ☐ **Note:** SBARF used – 4 pts
- ☐ **Note:** Deviations (NMTARA or process) documented – 6 pts
- ☐ **Note:** Narrative details match call – 8 pts
- ☐ **Note:** Corti ID documented – 6 pts
- ☐ **NMTARA:** correct for call – 4 pts

Flags

Customer Service Issue

- × Unprofessional conduct (lack of empathy, frequent interruptions, rude, dismissive, etc.)
- × Negative statements re: pt eligibility/qualifications for EMS or pt situation
- × Nurse statements disparaged EMS system or capabilities
- × Nurse's actions were not in alignment with contract-specific guidance
- × ALS escalation reasoning unclear
- × NMTARA 6 / Triage Not Complete reasoning unclear

Patient Safety Issue

- × Red flag symptoms
- × Did not utilize triage pathway or used inappropriate triage pathway
- × Care recommendation lower than NMTARA classification
- × Exclusion criteria met, continued triage

Keys to Success

Follow	Use	Verify	Document
Follow the conversation guide! It was designed to keep all of 911 NN on track!	Use the tools that are there to help you: Corti, Genesys tags, whisper, etc. <i>If it's unclear how, please ask!</i>	Verify HIPAA data The importance of this only grows as we work with clients and payers.	Document clinical reasoning for deviations Triage Not Completed - NMTARA 6 / Exclusion Criteria Care escalation above NMTARA expectations

Transportation Guide

Transport.Net

GMR's web-based solution for ride ordering

The plan is to utilize [Transport.net](#) for ride ordering on all contracts over time.

- 1st Contract: 911 CA, Santa Clara
- 2nd Contract: 911 WA, King County (GoLive 12.10.24)

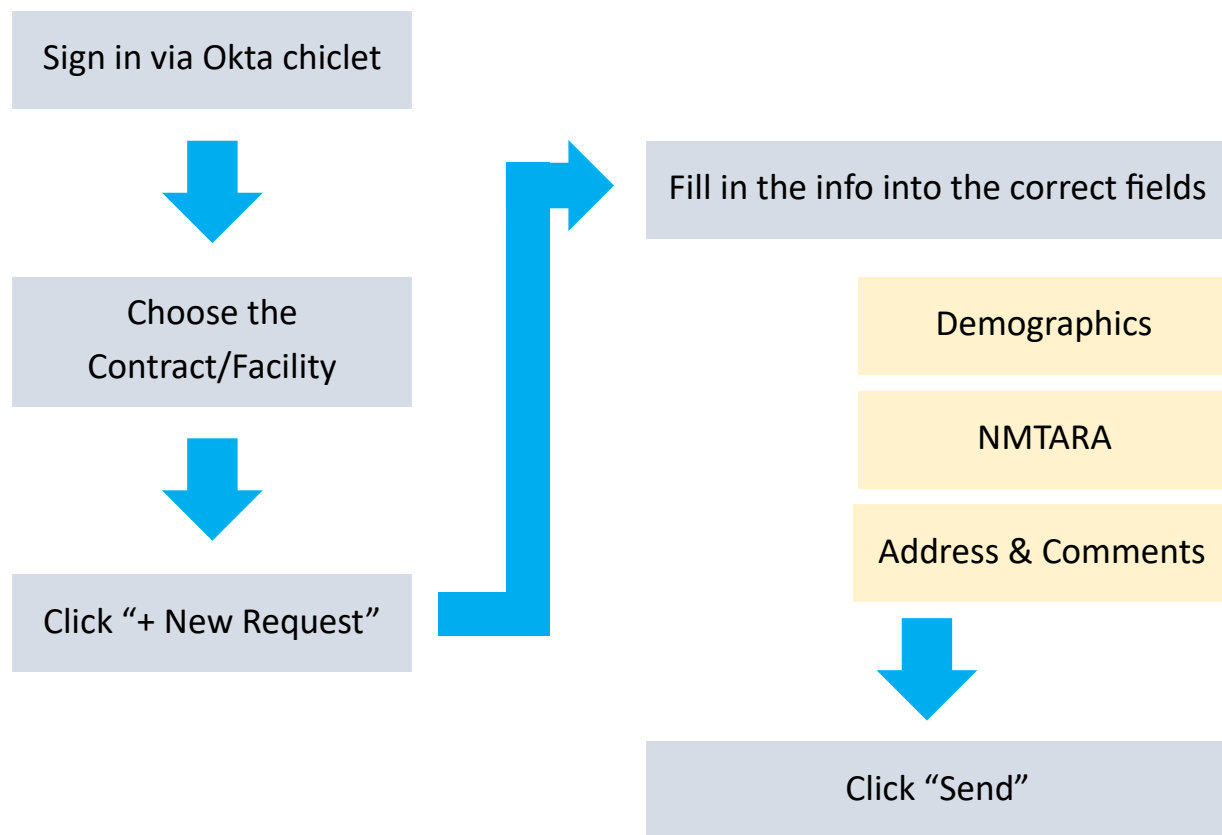
Right now, NN is only using Transport.Net for BLS ambulance requests

- Future state:
 - Rideshare Requests will shop Lyft and Uber for the lowest fare
 - One-click ordering from within Logis

No, we don't have an ETA

Workflow

CORTI will guide you in what to do.



Transport.Net Form

Clinical

[Collapse All Questions](#)

▼ Confirm the NMTARA Level - drop down ✕

Confirm the NMTARA Level - drop down

☐ NMTARA 0 (right now)

☐ NMTARA 1 (within 1 hour)

☐ NMTARA 2 (within 2-4 hours)

☐ NMTARA 3 (within 12 hours)

☐ NMTARA 4 (within 24 hours)

☐ NMTARA 5 (within 5 days)

☐ NMTARA 6 (unable to be triaged)

Scheduling

Pickup *

My Facility

Enter search query to find the Pickup Facility

Address selected is not geovalidated, please select a different address

Pickup Bed/Apt/Room

Pickup Bed/Apt/Room

Pickup Comments *

Pickup Comments

→ One Way

Pickup Date

Date

Pickup Time

Time

☒ Right Now

Attachments & Electronic Forms

Expand ▼

Send

Save Draft

Cancel

What's Next?

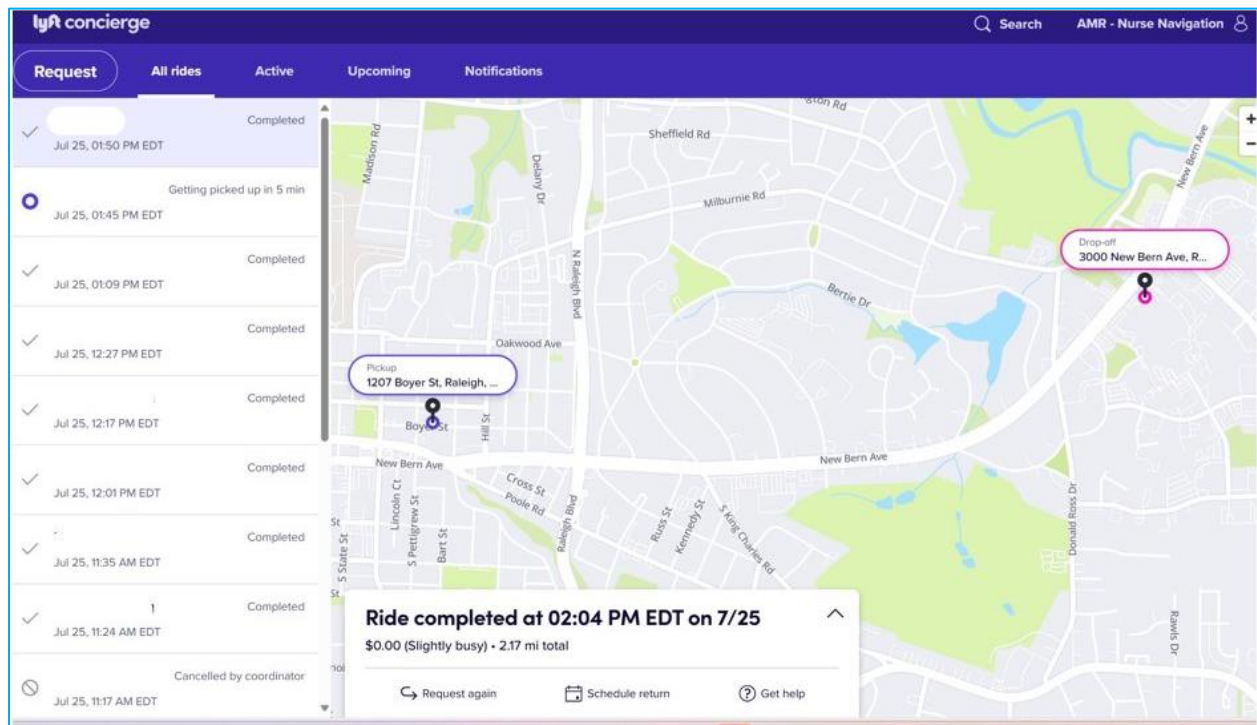
- Wait for a response: can take up to 8 minutes
 - Sends request to vendors and allows 4 minutes for reply.
 - If no response, sends again.
 - If still no response, you will get message that no vendors are available.
- If accepted: check against contract's guidelines in Corti.
 - If within guidelines, choose "BLS Transport arranged within guidelines" and follow Corti instructions.
 - If outside guidelines, choose "BLS Transport's ETA outside NMTARA guidelines," follow instructions.
- If not accepted: choose "BLS Transport not picked up by provider" and follow Corti instructions

Limitations

- Needs a numbered address
 - If your patient is at a street corner, ask what building they're in front of and look it up on Google Maps.
 - If your patient is on the side of the highway, use the backup option (this will vary by contract).
- Can take up to 8 minutes to get a final result
 - Sends request to vendors and allows 4 minutes to reply.
 - If no response, send again.
 - If still no response, will then notify that no vendors are available.

Lyft: Preferred Transportation in most contracts

Log in at the beginning of your shift and leave open for the shift.



1. You must enter the patient's first and last name, telephone number (does not need to be cell but it is preferred). Pick up address and drop off address should be entered manually instead of copy and paste.
2. Note for the driver: information that is important or helpful to the driver. "Rider is elderly and may take a few minutes." Or "Rider wearing a yellow hat."

The screenshot shows the 'Start ride request' form in the Lyft Concierge app. The form is titled 'Start ride request' and has tabs for 'Request', 'All rides', and 'Active'. Under 'Enter rider details', there are fields for 'First name*' (Barney), 'Last name*' (Fife), 'Rider's mobile phone number*' ((555) 555-5555), and 'Rider's preferred language' (English). A note states 'Rider will get text messages with ride details in this language'. Under 'Enter pickup and drop-off addresses', there are fields for 'Pickup address (US)*' (1950 W Exchange Pl, Tucker, GA) and 'Drop-off address (US)*' (1220 Caroline St NE Ste A 230, Atlanta GA 30307). A note shows the fare: '\$18.31 / 9.32 miles'. At the bottom, there is a link 'Share ride info with others (optional) Open'.

3. Select standard ride or wait and save if the patient needs more time to get ready.
4. Enter request notes (Internal note):
 - You must enter the contract and destination.
5. Ex: Seattle-ER, Knox-Pharmacy, Wake-Urgent Care, etc
6. Select Request Ride for driver information

Ride details for Barney Fife

Select date and time

Pickup date
Today, 7/28

Pickup time
Now

Select Lyft ride type


Lyft 4

within 15 min Wait & Save	\$14.99 12:35 PM - 12:44 PM
in 6 min Standard	\$15.82 12:35 PM
in 4 min Priority Pickup • Expires soon	\$18.31 12:33 PM
XL 6	\$25.89

Today, 7/28

Now

Select Lyft ride type


Lyft 4

within 15 min Wait & Save	\$16.49 12:36 PM - 12:46 PM
in 5 min Standard	\$16.99 12:36 PM
XL 6 in 5 min	\$28.61 12:36 PM

Displayed price is an estimate. Actual price may vary.

[View additional ride types](#)

Enter request notes (internal use only)

Internal note (optional)

Dekalb-UC

By selecting "Request ride," I'm certifying that I have the rider's consent for Lyft to use their information and to send SMS messages (rates may apply) for the ride.

Back

Request ride

Driver Details

- Patients usually receive a text with driver info but if they have not, provide name and car info with ETA.
- You **MUST** document the Ride ID in your journal as well as confirmation that your patient has reached their destination.

The screenshot shows the Lyft app interface for an upcoming ride. The top bar has 'Upcoming' and 'Notifications' tabs. Below, the 'Rider' section shows fields for 'Full name', 'Mobile phone number', and 'Ride type' (Lyft Standard (4 seats)). The 'Route' section displays the pickup location '2001 S Main St, Wake Forest, NC, United States' and the drop-off location '328 S Franklin St, Wake Forest, NC, United States'. A note states: 'Addresses may vary from the original request due to minor pickup and drop-off changes. For example, an address may vary if a rider is picked up across the street.' The 'Driver' section lists 'Name and number' (Naser, (555) 555-5555), 'Make and Model' (Chevrolet Midsize SUV), 'License plate' (USA123), 'Note to driver' (Blue shirt), 'Tip added' (\$0.00), and 'Additional tip'. The 'Request details' section at the bottom includes 'Coordinator name' (Wake County UC), 'Date and time' (7/28/25, 12:18 PM EDT), and a highlighted 'Ride ID' (1234567890123454).

The Uber Alternative

Same process, different platform.

The screenshot shows the Uber app interface for a 'New ride setup'. The left panel contains input fields for 'Who's riding?' (Rider first name, Rider last name, Rider's phone number, Member ID) and 'Plan ride' (One-way trip, Round-trip). The 'First leg' section includes a 'Pickup address' field with a search icon and the text 'Where from?'. At the bottom of the left panel are 'Save as draft' and 'Set up ride' buttons. The right panel shows a map of a city area with various streets and landmarks labeled, including 'HIGHLAND TERRACE', 'RESERVOIR HILL - MEADOW BROOK', 'OAK TREE', 'HAWTHORNE', 'ST. ELIZABETH', 'FOOTHILL BLVD', 'FRUITVALE AVE', 'PATTEN', 'PERALTA HACIENDA', 'FOOTHILL MEADOW PARK', 'La Clinica De La Raza', 'Bank of America', 'FoodMaxx', 'McDonald's', 'Restaurant Design Conc', 'St Dance Ballroom', 'SOLANO WAY', 'KENNEDY ST', 'E 10TH ST', 'E 20TH ST', 'E 21ST ST', 'E 22ND ST', 'E 23RD ST', 'E 24TH AVE', 'E 25TH AVE', 'E 26TH AVE', 'E 27TH ST', 'E 28TH AVE', 'E 29TH AVE', 'E 30TH AVE', 'E 31ST AVE', 'E 32ND AVE', 'E 33RD AVE', 'E 34TH AVE', 'E 35TH AVE', 'E 36TH AVE', 'E 37TH AVE', 'E 38TH AVE', 'E 39TH AVE', 'E 40TH AVE', 'E 41ST AVE', 'E 42ND AVE', 'E 43RD AVE', 'E 44TH AVE', 'E 45TH AVE', 'E 46TH AVE', 'E 47TH AVE', 'E 48TH AVE', 'E 49TH AVE', 'E 50TH AVE', 'E 51ST AVE', 'E 52ND AVE', 'E 53RD AVE', 'E 54TH AVE', 'E 55TH AVE', 'E 56TH AVE', 'E 57TH AVE', 'E 58TH AVE', 'E 59TH AVE', 'E 60TH AVE', 'E 61ST AVE', 'E 62ND AVE', 'E 63RD AVE', 'E 64TH AVE', 'E 65TH AVE', 'E 66TH AVE', 'E 67TH AVE', 'E 68TH AVE', 'E 69TH AVE', 'E 70TH AVE', 'E 71ST AVE', 'E 72ND AVE', 'E 73RD AVE', 'E 74TH AVE', 'E 75TH AVE', 'E 76TH AVE', 'E 77TH AVE', 'E 78TH AVE', 'E 79TH AVE', 'E 80TH AVE', 'E 81ST AVE', 'E 82ND AVE', 'E 83RD AVE', 'E 84TH AVE', 'E 85TH AVE', 'E 86TH AVE', 'E 87TH AVE', 'E 88TH AVE', 'E 89TH AVE', 'E 90TH AVE', 'E 91ST AVE', 'E 92ND AVE', 'E 93RD AVE', 'E 94TH AVE', 'E 95TH AVE', 'E 96TH AVE', 'E 97TH AVE', 'E 98TH AVE', 'E 99TH AVE', 'E 100TH AVE'.

7. Confirm the rider arrived at their destination
8. Document
 - Document Trip ID to the journal.
 - Example: Abc12345-abc1234-abc123456-abc123

